

Statement of Deficiency Report

Department of Health
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Pearl Youth Residence
1201 S. Proctor St., Tacoma WA 98405
Agency Name and Address

Kimberly Dozal
Administrator

Investigation
Inspection Type

Wednesday, September 20, 2023
Investigation Start Date

EJL2303
Investigator Number

2023-11257
Case Number

BHA.FS.61067737
License Number

Child Long Term Inpatient
Placement (CLIP)
BHA/RTF Agency Services Type

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

Deficiency Number and Rule Reference	Findings	Plan of Correction
WAC 246-341-0600 Individual rights. (1) Each behavioral health agency must protect and promote individual participant rights applicable to the services the agency is certified to provide in compliance with this chapter, and chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in	Based on interview, observation, and document review the facility failed to protect and promote individual patient rights; comply with patient rights listed in RCW 71.05; ensure that patients were treated with respect, dignity, and privacy; and ensure that patients were free from abuse, sexual harassment, and exploitation. Failure to ensure that patient rights are protected and that patients are treated with dignity and respect, may cause a systemic failure to ensure a safe and supportive treatment environment; exacerbate patients' mental health conditions; or lead to instances of patient abuse or neglect.	

compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (d) Be treated with respect, dignity and privacy, except that staff may conduct reasonable searches to detect and prevent possession or use of contraband on the premises or to address risk of harm to the individual or others. "Reasonable" is defined as minimally invasive searches to detect contraband or invasive searches only upon the initial intake process or if there is reasonable suspicion of possession of contraband or the presence of other risk that could be used to cause harm to self or others; (e) Be free of any sexual harassment; (f) Be free of exploitation, including physical and financial exploitation. (g) Have all clinical and personal information treated in accord with state and federal confidentiality regulations. WAC 246-341-1133 Evaluation and treatment services—Service standards. Evaluation and treatment services—Service standards. (1) Evaluation and treatment services are provided for	Findings included: Item #1 - The right to be free of sexual abuse, harassment, and exploitation. - Review of a policy titled, "General Client Rights", last revised 08/2022, showed the following: - Patients have the right to be treated with respect, dignity, and privacy. - Patients have the right to be free of sexual harassment; be free of exploitation; and have all clinical and personal information treated in accordance with state and federal confidentiality regulations. - Review of a policy titled, "Abuse and Sexual Misconduct Prevention", last revised 03/2023, showed the following: - The policy required adherence to safe, ethical, and legal standards for employee conduct. - That "no employee or other person has the authority to commit or fail to report such conduct". - That "when abuse or sexual misconduct is reasonably suspected or is reported by an alleged victim an investigation will occur." - That the investigative process would include reporting to the Department of Health, Law Enforcement and other entities as required by law and within mandated timelines. - If the alleged wrongdoer was a facility staff, human resources (HR) would also receive the report immediately. - It defined "abuse" as physical, sexual, emotional, or psychological actions that influenced another person, and included the blatant and intentional disregard for the safety or well-being of others. - It defined "sexual contact" as an "intentional touching of the lips, genitalia, thigh, breast, buttocks, or clothing or clothing covering those areas of a person done for the purpose to abuse,	
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individuals who are held for 120-hour detention or on 14-day, 90-day, or 180-day civil commitment orders according to chapters 71.05 and 71.34 RCW. (i) Residents must be protected from assault, abuse, and neglect. Suspected or alleged incidents of nonaccidental injury, sexual abuse, assault, cruelty, or neglect to a child must be reported to a law enforcement agency or to the department of children, youth, and families and comply with chapter 26.44 RCW. RCW 71.05.217 Rights—Posting of list. (1) Insofar as danger to the individual or others is not created, each person involuntarily detained, treated in a less restrictive alternative course of treatment, or committed for treatment and evaluation pursuant to this chapter shall have, in addition to other rights not specifically withheld by law, the following rights, a list of which shall be prominently posted in all facilities, institutions, and hospitals providing such services: (e) To have reasonable access to a telephone, both to make and receive confidential calls; (g) To have the right to individualized care and adequate treatment.	humiliate, harass, degrade, arouse, or gratify the sexual desire of any person.” h. It showed that sexual contact included physical conduct that was sexual in nature including touching or brushing; advances, comments, innuendoes, bullying, jokes, or gestures; invasion of sexual privacy; or by creating a sexually hostile environment characterized as comments or conduct that created an intimidating, hostile, or offensive environment. 3. Review of a policy titled, “Resident Rights-Grievances”, last revised 02/2021, showed the following: a. The facility would follow a structured approach to resolve grievances. b. The facility pledged to uphold the rights of patients to express grievances and ensure grievances could be made without fear of interference, coercion, or reprisal. c. It showed that if patients complain to a staff member, it would be handled by that staff member promptly, dependably, and courteously. 4. Review of a facility incident report, that was sent to DOH as a self-report, dated 08/24/23 at 04:56 PM, showed that Patient #1 alleged that Staff A, Behavioral Health Technician (BHT), put their hands down Patient #1’s pants in the laundry room several months ago. It showed that in response to the allegations Child Protective Services (CPS) and DOH reports were made; that Staff A was moved to a different unit; and that Staff A was expected not to be alone with patients. 5. During an interview on 09/21/23 at 09:15 AM, Staff D, Executive Director of Intensive Youth Services, stated that: a. On or around 08/19/23, Patient #1 disclosed during a “casual conversation” with a milieu staff that Staff A had inappropriately touched them. b. They couldn’t remember if Patient #1 told Staff E, Counselor,	
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	or Staff B, Milieu Manager. Staff D was unable to find documentation of the disclosure. c. Milieu staff weren't allowed inside the laundry room with patients because that area was not in view of the cameras. d. That there had been a group of patients (Patients #1- #4) that had all complained about Staff A "always looking down their shirts and touching them on the butt." e. After Patient #1 came forward, Patient #2 disclosed that Staff A told them "age is just a number." Patient #2 also said that Staff A put their hand up Patient #2's shirt during a one-on-one walk outside of the facility. f. The facility had no policy or procedure for staff taking patients on walks off-site. g. The incidents with Staff A hadn't prompted the facility to make any changes to their current process. h. Three grievance forms that were filed by other patients before Patient #1's disclosure. The forms were dated 07/18/23; 07/18/23 and 08/13/23, and listed concerns about Staff A's inappropriate conduct with patients. i. Unit supervisors had provided "some coaching" to Staff A in response to the grievances. Staff coaching wasn't formally documented anywhere. j. Communication amongst staff, including staff supervision activities, occurred mostly through Microsoft Teams. The investigations showed that facility staff utilized Microsoft Teams messaging to coordinate day-to-day facility operations, staff supervision and patient care activities, but that these messages are not permanently retained according to an established standard. k. Staff D did not report the touching incident with Patient #1 to law enforcement. l. Staff A was placed on administrative leave on 09/05/23. 6. Review of facility documents titled, "[Facility] Complaint Report", dated 07/18/23; 07/18/23 and 08/13/23, showed grievances filed by patients. The documents contained the	
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	following information: a. “[Staff A] has been making sexual comments like, “age is just a number” and [they] slap my ass. [They] also been looking down girls' shirts and has been making all of us uncomfortable.” b. “[Staff A] flirts with us kids. [They] tell them age is just a number, winks at them, touches them in places like their butts, gives them flowers, and overall is just really taken. [They] need to not work in a place where people have been sexually assaulted. Or with kids in general.” c. “[Staff A] makes us kids uncomfortable. [They] constantly make comments such as “age is just a number”, [they] wink at us in a non-joking way, stare at our chests, and touch us without permission (things like arm/ butt brushes, etc.).” d. All complaint reports reviewed showed that under “action taken” a staff documented “attempted to follow up”, and that there was “no response” from the grievant. e. All documents failed to contain the name(s) of the staff member(s) that responded to the grievances; the response date(s); if the complaint was resolved; or, if and what additional actions were needed. 7. Review of a Teams message channel titled “BHT Supervision”, dated 06/01/23 through 09/30/23, for Staff A, contained no documentation showing that concerns reported in the patient grievances were addressed by the facility through coaching or supervision. 8. During an interview with Investigators #1 and #2, Staff D, Executive Director of Intensive Youth Services and Staff B, Milieu Manager, on 01/31/23 around 01:00 PM, Staff B stated that patient could deposit complaint forms in a locked box in the laundry room, and that they [Staff B] checked the box once a week. Staff B stated once received, the forms were stored in the office they shared with Staff D. They stated that complaint forms “did not make it in the patient record or personnel file” and weren’t retained.	
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| | <p>9. A law enforcement document, titled "Department Incident Report", dated 09/03/23 at 8:45 PM, showed:</p> <ul style="list-style-type: none">a. On 09/03/23 law enforcement responded to the facility for an indecent liberties call [indecent liberties is a type of criminal charge], after Patient #1's legal guardian, Individual #3, had called 911 due to Patient #1 reporting to them they had been sexually assaulted by Staff A inside the facility.b. When law enforcement interviewed Patient #1, they stated that they reported sexual assault allegations to their counselor about a week before the 911 call.c. Patient #1 stated they felt dismissed and were told by facility staff that Staff A "just wasn't allowed to be alone with [patients]."d. Patient #1 believed law enforcement hadn't been notified of the allegations [by the facility] because no one from law enforcement came to speak with them until after Individual #3 called 911.e. Patient #1 believed the only way to get help was to call their legal guardian. <p>10. A law enforcement document titled "Department Incident Report", dated 09/05/23 at around 01:26 PM, showed:</p> <ul style="list-style-type: none">a. On 09/05/23 law enforcement responded to a lewd conduct call [lewd conduct is a type of criminal charge] from Patient #2 after they reported they'd been inappropriately touched by Staff A.b. Patient #2 stated inappropriate touching happened two-or-three times; that Staff A made inappropriate comments to them; and that facility staff had prevented Patient #2 from calling law enforcement in the past to report these incidents.c. Patient #2 stated that the facility wasn't taking incidents with Staff A seriously.d. Patient #2 had reported inappropriate words and behaviors by Staff A to facility staff several weeks ago, yet Staff A was still permitted to come to work. | |
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	11. A law enforcement document titled “Department Incident Report”, dated 09/05/23 at around 01:26 PM, showed: a. On 09/05/23 Staff D, Executive Director of Intensive Youth Services, was interviewed by law enforcement over the phone. b. Staff D stated that they first learned about Staff A’s inappropriate conduct toward patients in late August of 2023. c. The situation escalated on 09/03/23 when Patient #1 saw Staff A walking outside with other patients. d. Patient #2 reported [on/around 09/5/23] that Staff A had put their hands up Patient #2’s shirt while they had been on a walk together. e. Patient #3, disclosed [after 09/05/23] that Staff A had winked at them and told them they “deserved a real man”. 12. A law enforcement report titled “Department Supplemental Report”, dated 09/15/23, showed that Patient #1 was forensically interviewed. It showed the following: a. They reported that Staff A was “flirty” with female patients; would find ways to touch [patients’] bodies, primarily the buttocks area; and would make inappropriate comments to patients. b. Staff A’s behaviors made them feel uncomfortable. c. Facility staff weren’t taking patient complaints seriously; concerns about Staff A weren’t addressed; and Staff A continued to be allowed around patients unsupervised. d. On one occasion there was an inappropriate touching event with Staff A. When it was reported Patient #1 was told that cameras were checked and staff couldn’t discern if it had been accidental or non-accidental. There was no further action in response to the incident. e. On a second occasion, there was an alleged inappropriate touching event with Staff A in the facility’s laundry room. Patient #1 alleged that Staff A touched them in a sexual manner under the clothing.	
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	f. Patient #1 was fearful of disclosing the incident sooner as staff hadn't taken their prior complaints seriously and they felt they wouldn't be believed. g. When Patient #1 wanted to report to law enforcement, staff wouldn't let them use the phone because the non-emergency law enforcement number wasn't on their approved call list. h. Patient #1 stated they believed facility staff did everything they could to keep the incident with Staff A quiet. 13. Review of a law enforcement report titled "Department Supplemental Report", dated 09/15/23, showed that Patient #2 was forensically interviewed. During the forensic interview, Patient #2 disclosed: a. There was an alleged inappropriate, under the clothing touching incident with Staff A, when Staff A took them off-site on a walk. They stated Staff A's actions made them feel "absolutely disgusting". b. Patient #2 didn't feel like they could report the touching incident to staff because they wouldn't do anything. c. Staff B, Milieu Manager, told Patient #2 that they weren't allowed to use the phone to call law enforcement because the law enforcement number was not on their approved call list. d. Patient #2 distrusted staff at the facility, especially Staff B. e. Patient #2 stated that the facility did not take reports about Staff A's behaviors seriously. 14. Review of a Teams message dated 06/27/23, authored by Staff B, Milieu Manager, showed: "I'm getting some feedback about [Patient #3] and some concerns with boundaries/perception with [Staff A], one of the BHTs. [Patient #3] shared with a staff Sunday morning that [they think] [Staff A] is flirting with [them]." 15. Review of a document titled, "Individual Therapy Progress Note", dated 06/28/23, authored by Staff O, Therapist, showed: "We discussed some concerning things [Patient #3] reported	
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about a staff member. [Patient #3] shared that a BHT was making [them] uncomfortable.” The therapy note showed that Patient #3 agreed to meet with the BHT [later identified as Staff A], who they had reported for making them feel uncomfortable, “to work through their feelings.”

16. Review of a Teams message dated 06/28/23, showed that Staff B, Milieu Manager, messaged Staff A, BHT, to tell them about the concerns Patient #3 shared with Staff O, Therapist, during therapy. Staff B told Staff A that Staff O would be reaching out to follow up.

17. Review of a document titled, “Individual Therapy Progress Note”, dated 08/15/23, authored by Staff O, Therapist, showed that Patient #3 reported being touched inappropriately by another patient. It showed Patient #3 said they didn’t feel believed [by staff] and that was why they were delaying reporting or weren’t reporting incidents at all.

18. Review of a law enforcement report titled “Supplemental Report”, dated 09/28/23, showed:

- a. Staff D, Executive Director of Intensive Youth Services, was being re-interviewed by the detective assigned to the criminal investigation.
- b. Staff D stated that it [Issues with Staff A] started with a complaint made by Patient #4, and that complaints started coming in around July 2023. This statement contradicted earlier statements made by Staff D to law enforcement on 09/05/23, and to DOH on 09/21/23, regarding when they first became aware of complaints.
- c. There were four patients [Patients #1-#4] that had complained about Staff A making comments like “age is just a number”; alleging Staff A was touching their butts; giving patients flowers; and looking down patients’ shirts.

	d. There was a mediated conversation between Patient #3; Staff O, Therapist; Staff K, Unit Supervisor; and Staff A after Patient #3 alleged Staff A made inappropriate comments. e. The mediation was requested by facility administration. f. The mediated conversation and the outcome was going to be documented in a Progress Note. 19. Review of a law enforcement report titled “Supplemental Report”, dated 09/28/23, showed that Staff B, Milieu Manager, was re-interviewed by the detective assigned to the criminal investigation. During the interview Staff B stated that the allegations made by Patient #1 “caught [them] off guard” as they never “observed [Staff A] to have any boundary issues with any of the [patients].” When law enforcement asked Staff B about a potential 3rd victim [in addition to known victims [Patient #1 and Patient #2] who had made allegations against Staff A, Staff B stated that they were not aware of a 3rd victim. 20. Review of a Teams message authored by Staff B, Milieu Manager, dated 07/19/23 showed: “Following up about [Patient #2] saying the same thing [Patient #3] said about [Staff A]; [they] said [Staff A] smacked [their] butt; very concerning yet unsure of validity; will follow up with [Patient #2].” 21. During an interview with Investigators #1 and #2, Behavioral Health Investigators, and Staff O, Therapist, on 01/31/24 around 11:00 AM, Staff O stated that several grievances were filed against Staff A in a grievance lockbox kept in the laundry room. Staff O stated that none of the grievances had been signed by the grievant(s). Staff O said they were, “unable to nail it down” when asked about who filed the grievances. 22. Review of a Teams message exchange between Staff O, Therapist, and Staff U, Clinical Intern, dated 07/23/23, showed: “They have expressed concerns for sexual stuff as well”; “this f*****g guy”; “what [they] are describing is exactly what is	
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going on with [Patient #3]”; “Yes, and with [Patient #1]”; “it feels like it’s all [Patient #2’s] fault.”

23. Review of a Teams message exchange between Staff B, Milieu Manager; Staff P, Unit Supervisor; Staff F, Clinical Manager; and Staff T, Unit Supervisor, dated 09/04/23 showed a conversation regarding Patient #1’s phone use; that Patient #1 had talked about “a past resident who would want to come forward”, to which Staff P said, “I’m assuming [they’re] trying to drag [Patient #5] into this”.

24. During an interview with Patient #5, on 01/22/24 at 12:05 PM, they stated that:

- a. They were admitted to the facility between 03/2023 through 09/2023.
- b. Staff A was inappropriate with female patients. This was evidenced by them flirting and winking at Patient #3; telling patients that “age was just a number”; and that Staff A was doing things to other patients, including Patients #1 and #2.
- c. Facility staff knew about Staff A’s conduct but did not care.
- d. When Patient #5 told facility staff they were escalated over the situation with Staff A, they were put in the seclusion room.
- d. Patient #5 tried to tell their therapist, Staff O, “but [they] didn’t give a crap.”
- e. Milieu staff would joke with patients about flirting with staff.
- f. Staff R, BHT, talked to Staff A on the milieu and in front of patients about how they had a “crush” on an adolescent patient on the other unit.
- g. When patients filed grievances, milieu, clinical, and leadership staff would get mad and “talk crap about patients.”
- h. That Staff B, Milieu Manager, was unresponsive to grievances filed by patients.

25. During an interview with Staff P, Unit Supervisor, on 01/31/23, around 01:30 PM, they stated that they’d “heard about [Staff A]” twice and had looked at cameras but hadn’t

seen anything. Staff P stated that Patient #2 had complained about Staff A touching their butt. Staff P didn't remember the date of the incidents and stated there was no record.

26. Review of a Teams message authored by Staff P, Unit Supervisor, dated 07/30/23, showed that they found a torn-up note in the garbage and identified that the note was written by Patient #1. The note was put back together and posted on Teams. It showed: "It was a couple of weeks ago I don't know whether it was [their] hand or [their] arm but I was walking to the back table I don't know whether it was an accident or not but [they had] me uncomfortable I don't want [them] around DONT TELL anyone, not [patient] not [Patient #3] not [Patient #2] not [Patient #4] and not [Staff B] or any other staff!"

27. Review of a Teams message dated 09/04/23 showed a message exchange between Staff Q, BHT, and Staff AA, BHT, who asked "what happened yesterday". It showed Staff Q stating, "[Patient #1] saw [Staff A] take two female residents (and a male resident [Patient #1] didn't see) on a walk so [Patient #1] had [their legal guardian] call the police so [Patient #1] could make a police report." It showed Staff AA stated, "That is so f*****g cringe", to which Staff Q responded, "Yeppers."

Item #2 - The right to have reasonable access to a telephone.

1. Review of the facility policy titled, "General Client Rights", last revised 08/2022, showed that the policy outlined the rights of patients who receive services at the facility. It showed that it was the facility's responsibility to ensure that all patients were awarded their patient rights; that patient rights were protected; and that all staff adhered to these policies and procedures. It showed that all patients with involuntary treatment orders were also afforded rights protected under RCW 71.05.217, Rights.

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| | <p>2. Review of RCW 71.05.217 Rights, showed that insofar as danger to the individual or others is not created, each person involuntarily detained, [...], pursuant to this chapter shall have, in addition to other rights not specifically withheld by law to have reasonable access to a telephone, both to make and receive confidential calls.</p> <p>3. Review of law enforcement records titled “Department Supplement Report”, dated 09/05/23 and 09/15/23, showed that Patients #1 and #2 reported during their initial witness statements to law enforcement, and during their forensic interviews, that they were prevented from using the phone to call 911 and the non-emergency law enforcement number to report sexual misconduct by staff inside the facility.</p> <p>4. Review of law enforcement report, dated 09/26/23, showed that during the criminal investigation into Staff A, Staff B, Milieu Manager, was reinterviewed by the detective assigned to the criminal investigation in to Staff A.</p> <ul style="list-style-type: none">a. They stated patients at the facility were “technically detained” and were there by court order.b . Staff B stated that patients weren’t denied using the phone to call 911, but that the facility had “strict rules” related to phone use.c. They stated no one told Patient #1 they couldn’t use the phone, but that staff wanted to follow-up first to make sure they had the right type of support. <p>5. Review of law enforcement report, dated 09/26/23, showed that during the criminal investigation into Staff A, Staff F, Clinical Manager, was reinterviewed by the assigned detective. During the interview Staff F denied that the facility had prevented patients from using the phone to call 911. They stated that the facility “was going to have [them] wait to a more appropriate time of day.”</p> | |
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	6. During an interview with Individual #3, legal guardian to Patient #1, on 12/08/23 at 3:00 PM, they stated that: a. On or around 09/03/23 Patient #1 called them “frantically” and asked them to call law enforcement “right away” because they’d been sexually assaulted by a staff. b. Patient #1 told them that that facility staff said they weren’t allowed to call law enforcement. c. Facility staff informed Individual #3 after the incident that Patient #1 hadn’t been allowed to use the phone because they “weren’t asking for their phone privilege appropriately.” 7. During an interview with Staff B, Milieu Manger; Staff F, Clinical Manager; and Staff D, Executive Director of Intensive Youth Services, on 01/31/24 at 01:00 PM, patient access to the phone to report to DOH, CPS or Law Enforcement was discussed: a. Staff D stated that during the first instance of a patient wanting to make a report to an external entity, staff would try and gain a better understanding of what happened and help the patient in that stressful moment. b. Staff B stated that if there wasn’t an “active crime” to report, facility staff would want to first put supports in place to help the patient, and then reassess if the patient still wanted to make a report. c. Staff B stated they weren’t aware of any policy or procedure that outlined patient’s access to the phone. d. Staff D stated they also weren’t aware of any phone use policy. e. Staff F stated that all patients were offered an opportunity to participate [in making a report], but that staff would report regardless; that often no crime was committed in the moment that patients wanted to call, and therefore there was no urgency; and that “it was never a ‘no’ but maybe a ‘not right now’.”	
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f. Staff F stated that the facility did not have a phone use policy and procedure.

8. A Teams message exchange between Staff F, Clinical Manager, and Staff O, Therapist, dated 09/04/23, showed that they were still “trying to figure out why they [law enforcement] showed up”; “Did [Patient #1] use the phone when someone was not around?”; “[Patient #1] didn’t say anything new”; “Let’s talk outside of the building tomorrow”; and “Don’t put power in the narrative.”

9. Review of a Teams message exchange between Staff B, Milieu Manager; Staff P, Unit Supervisor; and Staff Q, BHT, on 09/5/23, showed:

a. Staff B stated the facility could deny Patient #1’s phone call because there had been no “recent crime” and no “imminent risk at this moment”.

b. Staff Q stated that it was the “it’s been happening since I’ve got here” for them [referring to allegations made against Staff A].

c. Staff Q stated, “[Patient #2] is currently trying to get their [legal guardian] to call 911 to have cops come [right now].” It showed that Staff P responded with, “it’s [their] second phone call, right?”.

10. Review of a Teams message exchange between Staff D, Executive Director; Staff O, Therapist; and Staff K, Unit Supervisor, dated 09/16/23, showed that facility staff denied Patient #1 use of the phone to contact law enforcement and CPS because Patient #1 had used their “two calls” for the day.

Item #3 - The right to be treated with dignity and respect

1. Review of the facility policy titled, “General Client Rights”, last revised 08/2022, showed that the policy intended to outline and describe the rights of patients who receive services at the

	facility. It showed that it was the facility's responsibility to ensure that all patients were awarded their patient rights; that patient rights were protected; and that all staff adhered to these policies and procedures. It showed that all patients would be treated with respect, dignity and privacy. 2. Review of the facility policy titled, "Resident Rights-Grievances", last revised 02/2021, showed that patients could make verbal and written complaints. It showed the facility pledged to uphold the rights of patients to express grievances and ensure grievances could be made without fear of interference, coercion, or reprisal. It showed if a patient complained, those complaints would be handled by that staff member promptly, dependably, and courteously. 3. Review of a Teams message authored by Staff S, Advanced Registered Nurse Practitioner (ARNP), sent to Staff D, Executive Director, on 07/19/23, showed: "[Patient #1] has a lot of complaints, of which maybe one fourth are valid concerns." The message showed that Staff D encouraged Staff S to tell Patient #1 to file a complaint in the [grievance] box in the laundry room. Staff S responded stating that Patient #1 was worried about staff reading them and retaliating. 4. Review of a Teams message exchange between Staff P, Unit Supervisor, and Staff B, Milieu Manager, dated 08/19/23 showed: "[Patient #1] just claimed to [Staff E, Counselor] [...], that about a month ago [Staff A] closed the door to the laundry room and put [their] hands down [Patient #1's] pants." 5. Review of a facility document titled, "Incident Report", dated 08/23/23, showed that Staff B, Milieu Manager, contacted CPS to report sexual assault allegations made by Patient #1. It showed that they then informed Staff A of the allegations. It showed that Staff A continued to work in their role as a BHT with unrestricted access to patients.	
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| | <p>6. Review of a Teams message exchange between Staff B, Milieu Manager, and Staff D, Executive Director, dated 08/24/23 showed that Patient #1's family asked that the "staff in question ([Staff A] but they don't know who)" not be scheduled in [Patient #1's] unit. Staff D responded that they thought the facility should honor their request while they go through the investigation.</p> <p>7. Review of a Teams message from Staff B, Milieu Manager, to Staff O, Therapist, dated 08/29/23, showed that Staff A was not to work on [Patient #1's] unit although "[they] are not banned from the unit".</p> <p>8. A DOH Investigative document, authored by Investigator #3, Health Care Investigator, titled, "Memorandum to File", dated 12/19/23, showed:</p> <ul style="list-style-type: none">a. Patient #3 was the first to come forward with a complaint about Staff A.b. Staff A acted "creepy" to them; commented on their body while they were in a bathing suit off site at a splash-pad; would frequently wink at them; and would touch them on the shoulder.c. They reported these behaviors to their therapist, Staff O, because they felt uncomfortable.d. As a result of their complaint they were "put in a room" with Staff A, by Staff O, and Staff K, Unit Supervisor, to talk about their complaint with Staff A present.e. They stated that other patients, including Patients #1, #2, and #5 had problems with Staff A's behavior. <p>9. Review of a Teams message authored by Staff B, Milieu Manager, dated 06/28/23, showed that they messaged Staff A, BHT, and told them about the complaint Patient #3 had made to their therapist.</p> | |
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	10. During an interview with Patient #5, on 01/22/24 at 12:05 PM, they stated that staff humiliated patients during room searches, including staff threatening to do “prison flips” of their rooms. They stated that some staff would pull out patients’ underwear during these searches and make fun of it in front of other patients and staff. 11. During an interview with Staff Z, a former employee, on 03/20/23 at 12:03 PM, they stated that: a. The facility hired unqualified and inexperienced staff to fill unit supervisor positions; that problematic issues, including staff bringing drugs and drug paraphernalia on the milieu, were not properly and consequently addressed by facility leadership; and, that staff showed no consistency in their engagement with the patients. b. Milieu staff did not understand the needs of the patient population or how their mental health conditions manifested behaviorally, therefore staff would take personal offense to patients’ behaviors. c. Milieu staff engaged in “gossiping” about other staff with patients on the milieu, which created a dangerous environment and power struggle that was ultimately to the detriment of the patients. 12. During an interview with Investigators #1 and #2, Staff D, Executive Director of Intensive Youth Services and Staff P, Unit Supervisor, on 01/31/23 around 01:00 PM, Staff P was asked about a progress note they had authored regarding a patient, Patient #6, telling them information about other patients on the milieu. a. Staff P stated that Patient #6 would often come to them to “share the tea” [tea is synonymous with gossip] about what was happening amongst the patients on the milieu. Patient #6 appeared “excited” and a little nervous to tell them about other patients trying to get Staff A fired.	
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b. Staff P identified the patients who had complaints about Staff A as “the mean girls club”.

13. A Teams message exchange between Staff P, Unit Supervisor, and Staff Q, BHT, on 08/23/23, showed:

a. Staff Q disagreed with Staff S, ARNP, opinions and directives [regarding patient care] as a medical professional.

b. Staff Q stated that they felt that it was very obvious that Patient #1 was “faking” and “highly exaggerating their episodes”.

c. They stated that Patient #1’s episodes were conveniently timed for when they didn’t want to do a thing; that “states of psychosis induced by anxiety or anything else very rarely last for 24 hours”; and that if Patient #1 was “genuinely in a state of psychosis” they wouldn’t be engaged in certain behaviors with other patients.

d. Staff Q also believed that Patient #4 was “faking and exaggerating their episodes”.

e. “I think operating under the assumption that every behavior both of them do is real and valid and giving them as much grace as [Staff S] wants us to would be antithetical to their treatment and negatively [affect] the treatment of the rest of the [unit] residents”.

f. It showed Staff P responded, saying that they were getting “others” to write this because otherwise, it was just them and, Staff K and Staff T, Unit Supervisors, saying that.

14. Review of a Teams message exchange between Staff B, Milieu Manager; Staff P, Unit Supervisor; and Staff B, BHT, on 09/05/23 showed these staff discussing under what circumstances they could take Patient #1’s door prop away; give them an “early bedtime”; and how to “[get] them off the milieu”.

a. The message exchange showed these three staff discussing under which circumstances Patient #1 could speak to law enforcement: “Staff told [Patient #1] that if police did show up

	to the facility that staff and /or supervisor [...] would go downstairs to have a conversation with police. If they requested [Patient #1] to make a statement [Patient #1] would be able to have a conversation with them but if not then [Patient #1] wouldn't be able to have a conversation." b. It showed that law enforcement responded to the facility and requested to take a statement from Patient #1. It showed that Staff Q was asked to leave the room so law enforcement could take the statement. c. Components of the message, authored by Staff Q, showed Staff Q describing how they "stayed on the other side of the [recreation] room door to listen in" and that they felt their hidden presence was justified because of Patient #1's "rights" as an ITA [involuntary treatment act] resident ("any statements you make may be used in involuntary treatment proceeding decisions" RCW 71.34.355 and 246-341-0805 WAC"). d. It showed that Staff Q captured detailed statements Patient #1 was making to law enforcement about the alleged sexual assault by Staff A. e. It showed that Staff Q posted the information they surreptitiously gathered from Patient #1's statements to law enforcement in the facility Teams chat. f. It showed that after Staff Q posted the initial message to Teams, they followed up with: "Just sent this to [Staff B, Milieu Manager] too." 15. Review of a Teams message posted by Staff P, Unit Supervisor, on 09/05/23, showed that they re-posted the message authored by Staff Q [described in finding #14 of this section] in Teams and added the following comment: " huge essay i know but that is the most detailed account i could find (shout out [Staff Q] for writing it) i already posted in supes [supervisor] chat but i don't think [Staff D] is in the clinical chat and i think [their] and BIG boss [Staff I, Chief Executive Officer] may be taking the lead on this".	
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	16. Messages posted to Teams by Staff B, P and Q on 09/05/23 [contents described in findings #14 and #15 of this section] were not restricted from other facility staff gaining access to view or share the information contained in them. a. Review of facility Teams messages, dated 06/01/23 through 09/30/23, showed that after Staff A was placed on administrative leave [on 09/05/23] they continued to interact with Teams messages, i.e., commenting on and liking photos, until at least 09/07/23. b. Review of policies and procedures provided by the facility showed that they did not contain policies directing how the facility would maintain reasonable control over information provided in Teams. 17. Review of the Integrated Licensing & Regulatory System Project (ILRS), a DOH information database, showed that on 09/05/23 Staff B, Milieu Manager; Staff P, Unit Supervisor; and Staff Q, BHT [now promoted to Unit Supervisor] were credentialed as agency affiliated counselors (AAC). Agency affiliated counselors are subject to Chapter 18.130 RCW- Regulation of Health Professions- Uniform Disciplinary Act (UDA), and WAC 246-16 – Standards of professional conduct. Actions and behaviors demonstrated by Staff B; P; and Q were not aligned with professional conduct expectations and did not provide and promote an environment that ensured the protection of patient rights, safety and wellbeing. 18. The gathering and distributing of sensitive patient information by Staff B, P and Q via Teams, which may also be available to staff on their personal phones, exposes protected health information to unauthorized access, compromising patient privacy and confidentiality. 19. Review of an email exchange between Investigator #1, Staff D, Executive Director of Intensive Youth Services, and Individual #2, Detective, on 01/02/24, showed: “[Staff A] has been	
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charged with two counts of Child Molestation in the 3rd Degree”.

Item #4 - The right for all clinical and personal information to be treated in accordance with state and federal confidentiality regulations.

1. Review of the facility policy titled, “General Client Rights”, last revised 08/2022, showed that the policy intended to outline and describe the rights of patients who receive services at the facility. It showed that it was the facility’s responsibility to ensure that all patients were awarded their patient rights; that patient rights were protected; and that all staff adhered to these policies and procedures. It showed that all patients have all information treated in accord with stated and federal confidentiality regulations.

2. During an interview with Staff Y, former BHT, on 03/13/24 at 2:41 PM, they stated, “everything is posted on Teams” and “they are big on social media, they all have it on their personal phones. Patient information posted on Teams can still be viewed on your personal phone.” They stated that not having Teams on your phone wasn’t really an option.

3. During an interview with Staff Z, former BHT, on 03/20/23 at 12:03 PM, they stated that everything is done via Teams and that facility staff have Teams on their agency issued phones as well as on their personal phones, where content such as patient photographs, patient identifying information and patient treatment information is shared in large volumes daily.

4. A Teams message exchange between Staff F, Clinical Manager, and Staff N, Case Manager, on 09/21/23 showed Staff F asking Staff N to “please get Teams off your personal cell phone so you are not on it so late at night”.

WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The administrator retains overall responsibility for responsibilities delegated to appropriate staff. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations; (b) There is sufficient qualified personnel to provide adequate treatment services and facility security; (c) All persons providing clinical services are appropriately credentialed for the clinical services they provide; (d) Clinical supervision of all clinical services including clinical services provided by trainees, students, and volunteers; (g) A written internal quality	Based on document review and interviews, the agency administrator and their designees failed to ensure that all applicable rules, policies, and ethical standards were adhered to; that administrative, personnel, and clinical policies and procedures were complied with; and that there were continuous efforts to improve the quality of care for patients. Failure to follow all applicable rules, policies, and ethical standards, and put forth continuous efforts to improve the quality of care for all patients may lead to disorganization and mismanagement of staff; increased risks of adverse events exacerbating patients mental health conditions; and the compromising of patient safety and the quality of care. Findings included: Item #1 - Adherence to administrative, personnel and clinical policies. 1. Review of the facility policy titled, “Abuse and Sexual Misconduct Prevention”, last revised 03/2023, showed the following: a. The policy would ensure adherence to safe, ethical, and legal standards for employee conduct. b. When abuse or sexual misconduct was reasonably suspected or reported by an alleged victim, an investigation would occur. c. That the investigative process would include reporting to the Department of Health, Law Enforcement and other entities as required by law and within mandated timelines. d. If the alleged wrongdoer was a facility staff, human resources (HR) would also receive the report immediately. e. The policy defined “abuse” as physical, sexual, emotional or psychological actions that influenced another person, and included the blatant and intentional disregard for the safety or well-being of others.	
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management plan, human resources plan or similarly specialized plan, as appropriate, is developed and maintained that: (i) Addresses the clinical supervision and training of staff providing clinical services; (ii) Monitors compliance with the rules in this chapter, and other state and federal rules and laws that govern agency licensing and certification requirements; and (iii) Continuously improves the quality of care in all of the following: (B) Use of evidence-based and promising practices; and (C) In response to critical incidents and substantiated complaints.	f. The policy defined “sexual contact” as an “intentional touching of the lips, genitalia, thigh, breast, buttocks, or clothing or clothing covering those areas of a person done for the purpose to abuse, humiliate, harass, degrade, arouse, or gratify the sexual desire of any person.” g. It showed that sexual contact included physical conduct that was sexual in nature including touching or brushing; advances, comments, innuendoes, bullying, jokes, or gestures; invasion of sexual privacy; or by creating a sexually hostile environment characterized as comments or conduct that created an intimidating, hostile, or offensive environment. 2. Review of the facility policy titled, “Critical Incidents”, last revised 03/2023, showed that the facility defined a critical incident as a negative event which was not desired or anticipated, that involved facility staff and patients. It showed that “allegations of abuse, neglect, fraud or exploitation” was listed as a critical incident. It showed that the facility would report any critical incident to the DOH within 48-hours or one business day; that the facility would report critical incidents to “other appropriate agencies” by the end of the next business day of the incident occurring; and that the completion of an internal incident report did not replace external reporting to law enforcement or the legal guardian. 3. During an interview with Investigators #1 and #2; Staff F, Clinical Manager; Staff D, Executive Director of Intensive Youth Services and Staff C, Quality Improvement Coordinator, on 01/16/23 at 09:00 AM, Staff F stated that often no crime was committed in the moments patients wanted to use the phone to make external reports. It is outside of Staff F’s scope to determine whether a crime was committed and limit or prohibit external reporting activities 4. During an interview with Investigators #1 and #2; Staff B, Milieu Manager; and Staff D, Executive Director of Intensive	
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	Youth Services, on 01/31/23 at 01:00 PM, Staff B stated that when there was “no active crime” that was being committed, staff would offer support and reassess if patients still wanted to make a report. It is outside of Staff B’s scope to determine whether a crime was committed and limit or prohibit external reporting activities. 5. Review of a clinical document for Patient #3, titled “Individual Therapy Note” dated 06/28/23, showed that they reported to their therapist that a milieu staff [Staff A] was making them uncomfortable by saying concerning things to them. 6. Facility documents titled, “[Facility] Complaint Report”, dated 07/18/23; 07/18/23 and 08/13/23, showed that additional complaints about Staff A’s inappropriate behaviors with patients were received by the facility. 7. Review of a “Milieu Progress Note”, dated 08/18/23 showed that Patient #1 told Staff E, Counselor, that Staff A put their hand down their pants and touched them inappropriately in the facility laundry room several months ago. 8. Review of a document titled, “Incident Report”, dated 08/23/23, showed that Staff B, Milieu Manager, made a report to CPS on 08/23/24 and that they informed Staff A of the allegations made by Patient #1. 9. Review of a document titled, “Incident Report”, dated 08/23/23 at 04:56 PM, showed that the facility reported the incident to the Department of Health. The required reporting window per WAC 246-341 for reporting critical incidents to DOH is 24 hours. The facility missed their required reporting window by 72 hours. 10. Review of a facility document titled, “Review Discussion Form”, dated 9/6/23, showed:	
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	a. On 08/25/23 Staff D, Executive Director of Intensive Youth Services, reported the incident involving Staff A to HR. b. In their report to HR, Staff D approximated patient complaints involving Staff A dating back to July of 2023. c. On 08/31/23 HR conducted an investigatory meeting with Staff A, where Staff A denied the allegations. d. During the HR meeting Staff A was informed that the outcome of the DOH investigation may lead to further action, including termination. e. Staff A was not placed on administrative leave till 09/05/23. 11. An email exchange between Investigator #1 and Staff C, Quality Improvement Coordinator, dated 02/29/24, showed that Staff A continued to be employed by the facility. Staff C stated that Staff A was still on paid administrative leave but that they did not have access to the building. 12. During an interview with Staff D, on 09/21/23, they stated that no one from the facility had contacted law enforcement to report the sexual assault allegations made by Patient #1 on 08/18/23. They stated that after a conversation with Staff L, Chief Administrative Officer, they realized that they had the responsibility of reporting allegations of sexual assault to law enforcement. Staff D did not state when they spoke to Staff L in relation to Patient #1 making allegations against Staff A. Staff D did not submit a report to law enforcement after their conversation with Staff L. 13. During an on-site visit to the facility on 01/16/23 Investigators #1 and #2 were taken to observe the facility's milieu and laundry room areas by Staff D, Executive Director of Intensive Youth Services. Investigators #1 and #2 observed the placement of the security cameras. During the tour, Staff D stated that camera footage is retained for 72 hours and then deleted. They stated it could only be kept if a specific request was made ahead of time. When asked if the recent allegations	
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made against Staff A by multiple patients had prompted the facility to reevaluate or change their security camera footage retention practices, Staff D stated that it had not.

14. During an interview with Staff Z, a former employee, on 03/20/24 at 12:03 PM, they stated that many problems at the facility were caused by a lack of leadership directive, and staff following policies and procedures inconsistently, if at all. Staff Z stated that staff following policies and procedures depended on the staff that were working and how those staff were feeling that day. Staff Z stated that administrators were rarely on-site at the facility and wanted to do everything from their phones; that newer milieu staff lacked adequate training; and that the facility was chronically understaffed.

Item #2 - Clinical supervision of clinical staff.

1. Review of the facility policy titled, "Program Structure", last revised 02/2021, showed that the program director [Staff D] would ensure that a written internal quality management plan and process was developed and maintained, to include ensuring the supervision and training of clinical staff.

2. Review of the facility policy titled, "Program Structure", last revised 02/2021, showed that the milieu manager was responsible for providing oversight of services to ensure a safe environment and was responsible for the supervision of assigned staff.

3. Review of a facility policy titled, "Clinical Supervision", last revised 07/2023, showed:

- a. Qualified mental health professionals would provide regular clinical supervision to all employees, volunteers, interns, and trainees providing direct treatment services.
- b. Clinical supervision would cover aspects of assessment, treatment planning, and service delivery.

	c. Clinical supervision would ensure compliance with professional standards. d. Case specific clinical supervision or consultation would be documented in the patient record. e. Supervision notes could be completed by either the clinical supervisor or staff member with approval from the clinical supervisor. 4. During an interview with Staff D, Executive Director of Intensive Youth Services, on 09/21/23 at 09:15 AM, they stated: a. There was one unit supervisor per shift and they were responsible for supervising and coaching the BHT's. b. Facility administration was soliciting feedback from staff with the goal of having "more formal sit-downs" for supervision. c. When staff showed concerning behaviors, usually other staff would report it if there was a problem. 5. During an interview with, Staff D, on 12/15/23 at 09:00 AM, they stated: a. Supervision activities and oversight of staff were documented and communicated through Teams channels. b. There was no formal process to ensure milieu staff received consistent supervision. c. The facility was working on a plan to ensure better supervision for staff. d. If a staff showed concerning behaviors a unit supervisor would post it in Teams for awareness and coaching purposes. e. When staff showed "more problematic conduct", issues were addressed by Staff B, Mileu Manager. f. Ongoing conduct issues would be referred to HR and staff may be put on a performance improvement plan (PIP) or be terminated. g. Information documenting patterns or trends that evidenced ongoing or repeated staff conduct issues would likely be captured in Teams. h. Teams was used to capture "real time information".	
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	i. They “monitored Teams in real time to notice trends”. j. They didn’t go back and look in Teams unless something “is a big deal” because they “just remembered things”. k. They had never considered monitoring trends of staff misconduct because “turnover was so high”. 6. During an interview with Staff B, Milieu Manager, on 01/31/24 at 01:00 PM, they stated that the supervision activities they provided to BHT’s and unit supervisors included casual monthly meetings and weekly meetings with the unit supervisors. 7. During an interview with Staff K, Unit Manager, on 01/31/24 at 01:00 PM, they stated that they were Staff A’s direct supervisor. They stated that for many of the BHT’s “this is their first job in mental health”; that the learning curve was steep; and that unit supervisors provided a lot of coaching, check-in’s and supervision during the first few months of that learning curve. These activities aren’t formally documented. 8. Review of a Teams message exchange, involving Staff B, Milieu Manager; Staff F, Clinical Manager; and, Staff D, Executive Director, dated 07/20/23, showed: a. Staff X, BHT, was talking to a former patient through a social media app. b. The facility had collected screenshots of photographic evidence of social media communications between Staff X and the former patient. c. Staff D stated, “the problem with this is that it wasn’t a one-time reach out”; “[...] the communication is no longer professional and sounds like a friendship. There is so much wrong with this.” d. Staff D believed the former patient had reached out to Staff X, indicating they wanted to die, and that the BHT hadn’t reported it to anyone.	
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	e. It showed the facility had no social media policy or staff training. f. Review of a message dated 07/20/23 showed that Staff K, Unit Supervisor spoke to Staff X who denied the allegations. g. A message dated 07/22/23, authored by Staff B, Milieu Manager, showed that the BHT was placed on administrative leave pending an investigation into their interactions with the patient post-discharge. h. The facility failed to report the staff member to the DOH per RCW 18.130.080 Unprofessional conduct; 246-16 Standards of professional conduct. i. The facility failed to report the incident per WAC 246-341-0420, (12) Reporting Critical Incidents. 9. Review of a Teams message channel titled “BHT Supervision”, contained a message authored by Staff K, Unit Supervisor, dated 08/12/23, that showed there had been an “ongoing conversation with [Staff V] about [their] boundaries with [patients] and how they could be perceived by others, ..., especially female [patients] who have been accusing male staff of misconduct.” It showed that Staff V was swapped from one unit to another, however; that they deliberately swapped back to do a one-to-one with a patient. It showed that Staff V appeared “coachable” in these situations, but continued to make choices that were perceived as questionable boundaries. 10. Review of a Teams message exchange dated 09/12/23 showed that Staff B, Milieu Manager, received a message with concerns about Staff V, BHT. It showed that concerns had already been reported to supervisors by the facility’s recreational therapist and involved the BHT deliberately going to a unit [they weren’t scheduled to work in] to see a patient. It showed Staff B’s response as “We’ve been working with [them] to have a better understanding of boundaries. I didn’t know Staff V was going to see [the patient] specifically but I was aware of the general concerns”.	
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11. A Teams message authored by Staff K, Unit Supervisor, dated 08/11/23 showed, "8/10 Reminded Staff R, BHT, about not showing [patients] their phone." A second message, dated 9/8/23, showed "9/8/23 Reminded Staff R, BHT, about not showing [patients] their phone."

Item #3 - Ethical standards to improve the quality of patient care.

1. Review of law enforcement report titled "Supplemental Report", dated 09/26/23, showed that Staff B, Milieu Manager, was reinterviewed by the detective assigned to the criminal investigation in to Staff A. During the interview, Staff B stated that a mediated conversation took place after Staff A learned about the complaints [made by Patient #3]. The report showed that law enforcement had been unaware of a mediated conversation involving Staff A as the facility had not provided records in response to law enforcements request [or in response to the search warrant served on 09/12/23].

2. Review of a law enforcement report, dated 09/28/23, showed that Staff D was reinterviewed by the detective assigned to the criminal investigation. During the interview Staff D stated that facility administration had requested mediation following the complaint [made by Patient #3]. They stated that the mediation resulted in a "Progress Note". During the interview, law enforcement highlighted that several documents, including the progress note capturing the mediated conversation, requested through the investigation and the search warrant appeared to not have been provided.

3. During an interview with Staff O, Therapist, on 01/16/24 at 11:00 AM, they stated:
a. They'd been the assigned therapist for Patients #1, #2, and #3.

	b. Patient #1 had not voiced concerns to them about Staff A prior to 08/18/23. c. They remembered facilitating the mediated conversation between Patient #3 and Staff A, in response to Patient #3 reporting concerns about Staff A during therapy. d. They could not recall why there had been a five-week delay from the time Patient #3 reported [06/28/23] that Staff A's conduct was making them uncomfortable, to the time the mediated conversation occurred [08/04/23]. e. They confirmed that on or around 06/28/23 they'd exchanged teams messages with Staff B, Milieu Manager, concerning Patient #3's complaints regarding Staff A. f. They were unable to explain what actions they had taken to report or address the concerns Patient #3 reported to them during therapy on 06/28/23. 4. Review of a document titled, "Milieu Progress Note", dated 08/04/23, showed that Staff O, Therapist, met with Staff A; Staff K, Unit Supervisor; and Patient #3. The document showed that "the goal of the meeting was for [Staff A] and [Patient #3] to have an opportunity to talk about the complaint [Patient #3] filed". The document showed that the "Service date" of the progress note was dated 08/04/23 and that it was signed by Staff O, Therapist, on 09/29/23. 5. Review of a document titled, "Milieu Progress Note", dated 08/18/23, showed that while checking in with Patient #1, they disclosed that "a male staff member put [their] hand in my pants in the laundry room". The document showed that the "Service date" of the progress note was dated 08/18/23 and that it was signed by Staff E, Counselor, on 09/28/23. 6. A Teams message authored by Staff D, Executive Director of Intensive Youth Services, to Staff B, Milieu Manager, dated 09/26/23 showed: "Who did the mediated conversations with [Staff A]?"	
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7. A Teams message authored by Staff O, Therapist, dated 09/26/23, showed: "Does anyone remember a rough date when [Patient #3's] concerns with [Staff A] arose? Me and [Staff K, Unit Supervisor] are going to put our heads together to do a progress note for our mediated conversation with them". Staff D, Executive Director: "Complaints came through 7/18 and 8/13 if that helps". Staff O: "Wanting to get a rough date. We met a week or two after the complaint. So, is the 7/18 [Patient #3's]?"

8. A Teams message exchange between Staff D, Executive Director, and Staff O, Therapist, dated 09/28/23, showed: Staff D: "Did someone do a note for the mediated conversation you had with [Staff A]?"

Staff O: "No. I could do one if it's needed. Me and [Staff K, Unit Supervisor] met with them so we could put our heads together and make one".

Staff D: "Yes, can you put something in there because I think DOH is going to ask what we have done and [I] want to send them what we have."

Staff O: "It will just be time-stamped for today which might be weird."

Staff D: "I'm not going to send it for now, but when we send the documents it doesn't print the time stamp."

Staff O: "K perfect".

9. A Teams message exchange between Staff D, Executive Director, and Staff E, Counselor, dated 09/28/23, showed Staff D messaging Staff E, saying "The detectives are wanting the documentation around your conversation about the sexual misconduct. Did you do 1 OR can you do one and then let me know?" The exchange showed that Staff E replied by asking Staff D if they wanted it in a milieu progress note or a therapy note, and stating that Staff O, Therapist, was out on vacation at the time. It showed Staff D's response as "Milieu progress note works for me". A follow-up message from Staff E, dated also for

	9/28/23, showed that they confirmed with Staff D that the document had been created. 10. A Teams message exchange between Staff B, Milieu Manager, and Staff O, Therapist, dated 09/29/23 showed: “[...] can’t find our original teams chat with [Staff A] but here is what I found in OUR chat [the message contained several images that that were not viewable].” Staff O: “Can you look to see if [Staff A] worked 8/3; I randomly picked a day but at least want to make sure [they] were here.” Staff B: “Not 8/3 but 8/4 though.” 11. A Teams message exchange between Staff D, Executive Director, and Staff O, Therapist, dated 09/29/23 showed: Staff D: “What was the date for the note; they are asking for it.” Staff O: “OMG I forgot... I will do it in a few minutes; the date of the note was 8/3 I would like to have someone look to make sure [Staff A] worked that day; I had to change it to 8/4.”	
WAC 246-341-0420 Agency policies and procedures. Each agency licensed by the department to provide any behavioral health service must develop, implement, and maintain policies and procedures that address all of the applicable licensing and certification requirements of this chapter including administrative and personnel policies and procedures. Administrative policies and procedures must demonstrate the following, as applicable: (8) Reporting and documentation of suspected abuse, neglect, or exploitation. A description of how the agency directs staff to report and	Based on interview and record review, the facility failed to develop, implement and maintain policies and procedures that address all of the applicable licensing and certification requirements. Failure to ensure that policies and procedures are developed, maintained and implemented may lead to ineffective treatment, misunderstandings, and potential harm to patients due to errors and lack of competency among staff. Findings included: 1. Review of the facility policy titled, “Critical Incidents”, last revised 3/2023, showed a critical incident was defined as a negative event which was not desired or anticipated and that involved facility staff and patients. It showed that “allegations of abuse, neglect, fraud or exploitation” was listed as a critical	

document suspected abuse, neglect, or exploitation of a child or vulnerable adult consistent with chapters 26.44 and 74.34 RCW. (9) Reporting of impaired practitioners in accordance with chapters 18.130 RCW and 246-16 WAC. (12) Reporting critical incidents. A description of how the agency directs staff to report to the department within 48 hours any critical incident that occurs involving an individual, and actions taken as a result of the incident. A critical incident is a serious or undesirable outcome that occurs in the agency including: (a) Allegations of abuse, neglect, or exploitation; (b) Death, including death by suicide; (c) Injuries resulting in admission to a hospital as an inpatient; (15) Individual rights. A description of how the agency has individual participation rights and policies consistent with WAC 246-341-0600. (16) Individual complaints. A description of how the agency addresses an individual's right to report an alleged violation of chapter 70.41, 71.05, 71.12, 71.24, or 71.34 RCW, and this chapter consistent with WAC 246-341-0605. (i) Agency program standards of prohibited conduct; and (ii) Actions to be taken in the event a staff member misuses alcohol or other drugs, including referral to a department-approved	incident. It showed that the facility would report any critical incident to the DOH within 48-hours or one business day; that the facility would report critical incidents to “other appropriate agencies” by the end of the next business day of the incident occurring. 2. Review of a policy titled, “Abuse and Sexual Misconduct Prevention”, last revised 3/2023, showed that “when abuse or sexual misconduct is reasonably suspected or is reported by an alleged victim an investigation will occur.” It showed that the investigative process would include reporting to the Department of Health, Law Enforcement and other entities as required by law within mandated timelines. It showed that if the alleged wrongdoer was a facility staff, HR would also receive the report immediately. 3. Review of facility documentation that included the self-report made to the DOH on 08/23/23; the report made to CPS on 08/23/23; interviews with facility staff; interviews with law enforcement; patient records review; and internal facility records review showed that the facility failed to follow their own critical incident reporting policy. 4. Review of a facility policy titled, “Mandatory Reporting” last revised 2/2023, showed: a. Facility staff were mandated to report potential instances of abuse, neglect, or exploitation, in accordance with RCW 26.44.030 and 74.34.035. b. Staff were required to report, or ensure that a report was made, when there was reasonable cause to believe that a suspected instance of abuse, neglect or exploitation had occurred. c. It showed, “Determination of whether to report suspected instances of abuse, neglect or exploitation will be made by an individual in consultation with a supervisor or program director.”	
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impaired practitioner or voluntary substance use monitoring program. RCW 26.44.030 Reports—Duty and authority to make. (1)(a) When any practitioner, [...] registered or licensed nurse, social service counselor, psychologist, [...] has reasonable cause to believe that a child has suffered abuse or neglect, he or she shall report such incident, or cause a report to be made, to the proper law enforcement agency or to the department as provided in RCW 26.44.040. RCW 26.44.040 Reports—Oral, written—Contents. An immediate oral report must be made by telephone or otherwise to the proper law enforcement agency or the department and, upon request, must be followed by a report in writing. RCW 74.34.035 Reports—Mandated and permissive—Contents—Confidentiality. (1) When there is reasonable cause to believe that abandonment, abuse, financial exploitation, or neglect of a vulnerable adult has occurred, mandated reporters shall immediately report to the department. (2) When there is reason to suspect that sexual assault has occurred, mandated reporters shall immediately report to the	d. It showed that reports would be made to the appropriate authorities, such as the Department of Social and Health Services or law enforcement, after consultation with a supervisor or program director. e. It showed that reports made would be documented in the patient record. f. It showed the facility would create an internal incident report and capture the agency and individual who accepted the report and detailed contact information. g. Review of the facility policy and procedure showed that it was not in compliance with applicable state mandates governing mandated reporting requirements. 5. During an interview on 09/21/23 at 09:15 AM, Staff D, Executive Director of Intensive Youth Services, stated that an allegation of staff sexual misconduct was made by Patient #1 on or around 8/18/23. They stated that the facility notified CPS and DOH on 8/23/23. Staff D stated that no one from the facility had notified law enforcement. 6. During an interview with Staff D, on 09/21/23 at 09:00 AM, they stated that the facility did not have a policy and procedure for staff taking patients on walks off-site. They stated: a. “If a girl is crushing on a staff member, we will not allow the staff member to go on walks with the girl and tell them to make sure they always stay on camera.” b. That the facility didn’t prohibit opposite-gendered staff, e.g., a male staff to go on a walk with a female patient, because that could be viewed as a discriminatory practice because some staff and patients regarded themselves as gender fluid. This also made creating a policy and procedure difficult. c. That incident with Staff A had not prompted the facility to make any changes to their existing practices of allowing staff to take patients off-site.	
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appropriate law enforcement agency and to the department.	7. Review of law enforcement records and internal facility records showed that Patients #1 and #2 repeatedly reported they were prevented from using the telephone to call 911 and the non-emergency law enforcement number to report sexual misconduct by a staff inside the facility 8. During an interview with Staff D and Staff F, Clinical Manager, on 01/16/24, Staff F stated that they did not have a phone use policy or procedure for patients. 9. During an interview with Staff D and Staff B, Milieu Manager, on 01/31/24, Staff B stated that they were not aware of a policy that outlined when patients could use the phone. Staff D stated they also weren't aware of an existing policy.	
WAC 246-341-0515 Personnel—Agency staff requirements. Each behavioral health agency must ensure that all of the following staff requirements are met: (1) All staff providing clinical services are appropriately credentialed for the services they provide, which may include a co-occurring disorder specialist enhancement. (2) All staff providing clinical services receive clinical supervision.	Based on record review and interviews, the facility failed to ensure that staff providing clinical services were appropriately credentialed for the services they were providing, and that all staff providing clinical services received supervision. Failure to ensure that staff providing clinical services are appropriately credentialed and receive clinical supervision may lead to inappropriate treatment interventions and outcomes; a worsening in the patient's condition; a compromise of safety protocols and ethical standards; and place vulnerable patients at risk of harm. Findings included: 1. Review of a facility policy titled, "Clinical Supervision", last revised 7/2023, showed: a. Qualified mental health professionals would provide regular clinical supervision all employees, volunteers, interns, and trainees providing direct treatment services. b. Clinical supervision would cover aspects of assessment, treatment planning, and service delivery.	

	c. Clinical supervision would ensure compliance with professional standards. d. Case specific clinical supervision or consultation would be documented in the patient record. e. Supervision notes could be completed by either the clinical supervisor or staff member with approval from the clinical supervisor. 2. Review of a facility document titled, “Job Description”, dated 12/07/22, for Staff A, Behavioral Health Technician (BHT) showed: a. Duties included providing direct patient care services such as implementing individualized care specific to patient needs and strengths; establishing and maintaining a therapeutic relationship with patients; and ensuring the safe care and supervision of patients. b. Staff A was required to obtain or maintain a Washington State credential. 3. Review of the DOH database ILRS showed that Staff A was credentialed as a Counselor Agency Affiliated Registration and was supervised by the facility. 4. Review of a facility document titled, “Job Description”, dated 01/30/23, for Staff K, Unit Supervisor showed: a. Duties included monitoring staff [unit supervisors supervise BHT’s] for milieu issues and be responsible for the management of patients; ensure that a safe environment for patients was maintained; and ensure that staff maintained ethical and professional standards of practice. b. Staff K was required to obtain or maintain a Washington State credential. c. Review of the DOH database ILRS showed that Staff K was credentialed as a Counselor Agency Affiliated Registration and was supervised by the facility	
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	5. Review of a facility document titled, “Job Description” dated 04/10/23, for Staff B, Milieu Manager, showed: a. Duties included ensuring that a safe and structured environment for patients was maintained; applying crisis intervention skills to ensure patient safety; and promoting ethical decision making and appropriate boundaries to safeguard patient welfare. b. The milieu manager competencies included recognizing signs of burnout to maintain staff well-being and prevent errors. c. Staff K was required to obtain or maintain a Washington State credential. d. Review of the DOH database ILRS showed that Staff B was credentialed as a Counselor Agency Affiliated Registration. Their credential was not supervised by the facility, but by the Department of Health and Social Services 6. Review of a facility document titled, “Job Description”, dated 10/31/22, for Staff E, Counselor, showed: a. Staff E was responsible for providing individual and family counseling and for completing all necessary documentation to include progress notes, treatment plan, discharge summaries, consultation notes and assessments. b. The counselor position required a bachelor’s degree and one year of experience working with children and families. c. Staff E was required to obtain or maintain a Washington State credential. d. Review of the DOH database ILRS showed that Staff E was credentialed as a Counselor Agency Affiliated Registration and was supervised by the facility. 7. During an interview with Staff E, Counselor, on 01/16/24, they stated: a. They currently had a bachelor’s degree and were credentialed as an agency affiliated counselor (CAAR). b. They had worked as a case manager before being promoted to counselor.	
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	c. Their duties as a counselor included providing individual and family therapy; writing therapy notes, crisis plans and treatment plans; and completing CSSR's [Columbia Suicide Severity Rating Scale] for patients. d. Their supervisor, Staff F, Clinical Manager, signed off on CSSRs, but otherwise "looks over the notes but doesn't sign off on them". e. They had monthly supervision with Staff F and also met one-on-one, but the one-on-one meetings were not formally documented. 8. Review of a facility document titled, "Job Description", dated 08/02/22, for Staff O, Therapist, showed: a. Staff O was responsible for providing individual and family therapy and coordinating and documenting therapy services for all patients served. b. Staff O would maintain a patient care environment of safety and quality; manage crisis situations; utilize treatment plan interventions as a way to help patients meet treatment goals and objectives; and adhere to treatment program procedures. c. Staff O would ensure ethical and professional standards of practice were maintained. d. The therapist position required Staff O to have a master's degree and two years' of relevant experience. e. Staff O was required to obtain/maintain a WA State credential or license. f. Review of the DOH database ILRS showed that Staff O held active credentials as a Mental Health Counselor Associate (MHCA) and as a Social Worker Associate Independent Clinical License (SWAIC). 9. During an interview with Staff O, Therapist, on 01/16/24, they stated that in August of 2023, they transferred Patient #1 to Staff E, Counselor, for individual therapy. They stated there was no record that captured the justification for the transfer.	
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	10. Review of individual service records showed that 2 of 4 patient (Patient #1 and #4) records contained “Individual Therapy Progress Notes” that were authored and signed by Staff E, Counselor. Records reviewed were dated 06/12/23 through 12/11/23. a. The records showed that Staff E was providing individual therapy to patients. The records showed that individual therapy progress notes for Patients #1 and #4 did not contain supervisory signatures. b. Individual therapy progress notes contained in Patient #1’s clinical record, dated 09/29/23 through 10/29/23 showed that Staff E signed therapy notes with a title of “Case Manager.” 11. During an interview with Staff D, Executive Director of Intensive Youth Services, on 09/21/23; 12/15/23 and on 01/16/24, they stated that the facility currently did not have formal supervision for clinical staff that included BHTs, Unit Supervisors, or the Milieu Manager, in place.	
WAC 246-341-0640 Individual service record content. A behavioral health agency is responsible for the components and documentation in an individual's individual service record content unless specified otherwise in certification or individual service requirements. (1) The individual service record must include: (d) Individual service plan that: (ii) Addresses issues identified in the assessment and by the individual or, if applicable, the individual's parent(s) or legal representative; (iii) Contains measurable goals or objectives and interventions;	Based on interview and document review, the facility failed to ensure that the individual service record contained all required components and documentation. Failure to ensure that the individual service record is complete and contains all required documentation may lead to vulnerable patients continuing to be subjected to harmful situations; legal complications for patient receiving court ordered treatment; or, causing patients to feel a diminished trust in the services that are being provided if treatment plans or mandated reporting requirements aren’t followed. In addition, failure to ensure that patient encounters are documented in such a manner that clinical staff can rely on their independent recollection of the date; the time; the duration; and clinically significant behaviors demonstrated by the patient, may also lead to undesirable patient outcomes, including: diminished standards of patient care; a failure to capture clinically relevant details of patient	

(iv) Must be mutually agreed upon and updated to address changes in identified needs and achievement of goals or at the request of the individual or, if applicable, the individual's parent or legal representative; (2) When the following situations apply, the individual service record must include: (b) Documentation that any mandatory reporting of abuse, neglect, or exploitation consistent with chapters 26.44 and 74.34 RCW has occurred. (c) If treatment is court-ordered, a copy of the order. (f) Progress and group notes including the date, time, duration, participant's name, response to interventions or clinically significant behaviors during the group session, and a brief summary of the individual or group session and the name and credential of the staff member who provided it. WAC 246-341-1133 Evaluation and treatment services—Service standards. Evaluation and treatment services—Service standards. (k) When suspected or alleged abuse is reported, the individual service record must reflect the fact that an oral or written report has been made to the child protective services of the department of children, youth, and families, or to a law enforcement agency within the timelines identified in chapter	encounters; an inability to ensure the quality of services provided; unreliability of clinical records; and could call in to question the ethical integrity of the clinician, or the facility promoting such practice. Findings included: Item #1 - Treatment Plans 1. Review of four clinical records showed that 2 of 4 (Patients #1, #2 and #3) individual service records did not contain a start date and an end date, or applicable DSM-V diagnostic criteria, codes or descriptors. Records reviewed showed that the individual service plan included sections for this information, and that these were left blank. 2. Review of the individual service record for Patient #2 showed that it contained a document titled “Nursing Assessment” dated 04/11/23. The document showed that Patient #2 disclosed a history of using substances. It showed that they were asked to complete substance use disorder (SUD) treatment while admitted to the facility. 3. Patient #2’s individual service record did not contain a referral for a substance use disorder (SUD) assessment. It did not contain documentation that showed Patient #2 was offered services or received services to address their substance use. Item #2 - Documentation of mandatory reporting events. Review of four clinical records showed that for 3 of 4 (Patients #1, #2 and #3) the individual services records did not contain documentation that a mandatory report of abuse, neglect or exploitation was made. Item #3 - Court orders	
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26.44 RCW. This note must include the date and time that the report was made, the agency to which it was made, and the signature of the person making the report. Contents of the report need not be included in the individual service record.	Review of four clinical records that 3 of 4 patients (Patients #1, #2 and #3) were Involuntary Treatment Act (ITA) patients. The records reviewed showed that they did not contain court orders. Item #4- Progress notes capturing responses to interventions or clinically significant behaviors. 1. Review of a “Milieu Progress Note”, for Patient #1, showed it was authored by Staff E, Counselor; had a service date of 08/18/23; and was signed on 09/28/23. 2. Review of a “Milieu Progress Note”, for Patient #3, showed it was authored by Staff O, Therapist; had a service date of 08/04/23; and was signed on 09/29/23. 3. Review of facility Teams messages, dated 06/01/23 through 09/30/23, showed that Staff E and Staff O were prompted by Staff D, Executive Director of Inpatient Youth Services, to recreate and backdate documents to be submitted as part of the individual service record content for Patients #1 and #3. 4. Review of facility Teams messages, dated 06/01/23 through 09/30/23, showed that Staff D asked Staff E and Staff O to create these records in response to law enforcement and Department of Health records requests. 5. Review of facility Teams messages, dated 06/01/23 through 09/30/23, showed that Staff E, Counselor, and Staff O, Therapist, were unable to recall the date; time; or duration of the patient contact. It showed that they required additional information by Staff D, and Staff K, Unit Supervisor, and Staff B, Milieu Manager, to recreate the progress notes to be added to the individual service records for Patients #1 and #3.	
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WAC 246-341-1133 Evaluation and treatment services—Service standards. (2) An agency providing evaluation and treatment services for youth must be a contracted child long-term inpatient treatment facility (CLIP), except as specified in subsection (4) of this section. The CLIP facility must develop a written plan for assuring that services provided are appropriate to t (h) Disciplinary policies and practices must be stated in writing and all of the following must be true: (i) Discipline must be fair, reasonable, consistent, and related to the behavior of the resident. Discipline, when needed, must be consistent with the individual treatment plan. (ii) Abusive, cruel, hazardous, frightening, or humiliating disciplinary practices must not be used. Seclusion and restraints must not be used as punitive measures.	Based on interviews and records review, the facility failed to ensure that discipline was fair, reasonable and consistent, and that seclusion was not used as a punitive measure. Failure to ensure that the facility exercised fair, reasonable, and consistent disciplinary practices, including not utilizing seclusion as a punitive measure, may lead to physical and psychological harm; constitutes a violation of patient rights; and could create reluctance to engage in treatment or seek help in the future. Findings included: 1. Review of a facility document titled, “Resident Handbook”, dated 02/2022, showed that “Off Programming” was defined as “ Your MDT [multidisciplinary team] has determined you are unsafe to be in the day use area. You must remain in room (or designated area) for a certain amount of time, usually for 24 hours. All your needs must be met by your contact, and you must be escorted to bathroom. You will be required to knock for staff and wait for them to access your needs.” 2. Review of the clinical record for Patient #4, showed: a. Patient #4 was admitted to the facility on 06/06/23 as a voluntary patient. b. The record contained a milieu progress note dated 08/13/23, that showed that the patient informed staff that they wanted to demand discharge. It showed they later on the same day they asked for their demand not to be processed. c. The document showed “staff let [Patient #4] know that even though it [discharge] wasn’t put though it still counts as asking for discharge and [they] would be Off Programming (OP) for the rest of swing shift.” 3. A Teams message exchange between Staff K, Unit Supervisor; Staff F, Clinical Manager; Staff P, Unit Supervisor; and Staff S, ARNP, dated 08/03/23, showed:	
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	a. Patient #4, a voluntary patient, had demanded to be discharged. It showed that Patient #4 then rescinded their demand for discharge. b. Staff S wrote:” Did anyone tell [Patient #4] that a DCR [designated crisis responder] would be coming to assess [them] and that due to [their] recent self-harm and blade to [their] neck indicating suicide risk that [they] would be likely detained for 6.5 months? Please educate [Patient #4] that voluntary treatment is [a] far better option than involuntary.” c. A response by Staff P showed, “I let [them] know didn’t go into many details but [they] already want to rescind.” d. Staff F wrote, “I’m glad [they] rescinded! My guess is [they] will do this a lot during treatment.” e. Staff K responded, “If it becomes a struggle, we could always do what we did with [another patient] where they were OP [off programming] for 24 hours any time they demanded even if they rescind to prevent them from doing it unless they were serious”. f. Staff F responded, “That’s a good idea if we get to that point.” 4. During an interview with Staff Z, former BHT, on 03/20/23 at 12:03 PM, they stated: a. When voluntary patients demanded for discharge, they were “automatically confined in their room and be off programming (OP) until they rescinded.” b. Facility staff justified taking this action due to needing time to call the designated crisis responder (DCR) to have the patient legally detained. c. When patients demanded to be discharged and were OP, they weren’t allowed to call anyone, except family on certain occasions. d. “In the [...] years I was there I’ve never seen a kid hold out and make it through. They’d always rescind.” e. When asked if OP was a form of seclusion, they stated “absolutely it is”, and that when patients were OP, they had to stay in their rooms with the door closed; they would get in	
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	trouble if they opened their door; and they had to rely on staff to take them to use the bathroom or for any of their other basic needs to be met.	
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Plan of Correction Instructions

Introduction

We require that you submit a plan of correction for each deficiency listed on the statement of deficiency form. Your plan of correction must be Submitted to DOH within fourteen calendar days of receipt of the list of deficiencies.

You are required to respond to the statement of deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

Descriptive Content

Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates

The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total

correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring

Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC

Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies, you will be sent a letter detailing why your POC was not accepted.

Questions?

Please review the cited regulation first. If you need clarification or have questions about deficiencies, you must contact the investigator who conducted the investigation.

Corrective Action Plan:
Pearl Youth Residence
DUE: 5/16/24

DOH Investigation: 2023-11257 BHA
BHA.FS.61067737

Item #1 – the right to be free of sexual abuse, harassment, and exploitation.

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0600 Individual Rights. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (e) Be free of any sexual harassment; RCW 71.05.217 Rights—Posting of list. (1) Insofar as danger to the individual or others is not created, each person involuntarily detained, treated in a less restrictive alternative course of treatment, or committed for treatment and evaluation pursuant to this chapter	Reporting Grid (circumstances to report and timeframes) has been updated to include CPS (under new regulations) (see attachment: Reporting Grid 2.24)	Complete February 2024	QI Coordinator ED of Intensive Youth Services	By 6/15/24, QI Coordinator will review the timeliness of reports for a randomly selected two week period and report to the ED of Intensive Youth Services.

shall have, in addition to other rights not specifically withheld by law, the following rights, a list of which shall be prominently posted in all facilities, institutions, and hospitals providing such services:				
WAC 246-341-0510 Personnel—Agency record requirements. A behavioral health agency must maintain a personnel record for each person employed by the agency. (1) The personnel record must contain all of the following: (a) A signed position description. (b) A signed and dated commitment to maintain patient (individual) confidentiality in accordance with state and federal confidentiality requirements. (c) A record of an orientation to the agency within 90 days of hire that includes all of the following: (i) An overview of the agency's policies and procedures. (ii) Staff ethical standards and conduct, including reporting of unprofessional conduct to appropriate authorities. WAC 246-341-0600 Individual Rights. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not	Pearl Youth Residence has added training in “Boundaries” & “Ethics” training specific to PYR. All new staff will be trained within their on-boarding (3 classroom days and 4 shadow shifts before working a shift) and annually after.	Complete April 2024	ED of Intensive Youth Services PYR Director	By 10/15/24, QI Coordinator will review completion of on-board training to determine if “Boundaries” and “Ethics” has been completed and report to the ED of Intensive Youth Services. QI Coordinator will meet with ED of Intensive Youth Services and the PYR Director to review onboarding training process by 2/15/25

specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (e) Be free of any sexual harassment;				
WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The administrator retains overall responsibility for responsibilities delegated to appropriate staff. (3) The administrator must delegate to a staff person the duty and responsibility to act on the administrator's behalf when the administrator is not on duty or on call. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations;	The process is in place and the procedure will be updated for any staff member alleged of engaging in sexualized behaviors.	5/31/24	Executive Director of Intensive Youth Services.	QI Coordinator will review PYR incident reports made between 6/1/24 and 6/14/24 to ensure the process is being followed by 6/15/24

	A Procedure will be written for staff taking residents on outings. Any changes to the policy will be communicated to the leadership team to implement.	6/30/24 7/15/24	Executive Director of Intensive Youth Services.	QI Coordinator will review the procedure by 8/1/24 QI Coordinator will review five outings that took place between 7/20/24 and 8/20/24 to determine if procedure was followed. By 8/30/24 RTF policies will be reviewed every two years.
RCW 71.05.217 Rights—Posting of list. NOTE: this section may be amended	Client Rights are posted on Milieus.	Complete April 2024	PYR Director	PYR Director will do two random checks to see the rights are still posted by 5/31/24 and 7/15/24
WAC 246-341-0600 Individual rights. (1) Each behavioral health agency must protect and promote individual participant rights applicable to the services the agency is certified to provide in compliance with this chapter, and chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in Based on interview, observation, and document review the facility failed to protect and promote individual patient rights; comply with patient rights listed in RCW 71.05; ensure that patients were treated with respect, dignity, and	The grievance process was updated in March 2024. The manager or director checks the complaint boxes once a week. The policy for PYR grievances will be updated to include more details of processing resident complaints. Policy update will be communicated to the PYR leadership team and staff will be trained how to respond and document.	Process - March 2024 Policy – 6/30/24 Training by 7/20/24	ED of Intensive Youth Services	Review of complaints made between 7/1/24 will occur every 6 months for one year by October 1, 2024 to ensure the complaints and grievances are being addressed according to the policy – Director, Exec. Director and QI Coordinator

privacy; and ensure that patients were free from abuse, sexual harassment, and exploitation. Failure to ensure that patient rights are protected and that patients are treated with dignity and respect, may cause a systemic failure to ensure a safe and supportive treatment environment; exacerbate patients' mental health conditions; or lead to instances of patient abuse or neglect. compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (d) Be treated with respect, dignity and privacy, except that staff may conduct reasonable searches to detect and prevent possession or use of contraband on the premises or to address risk of harm to the individual or others. "Reasonable" is defined as minimally invasive searches to detect contraband or invasive searches only upon the initial intake process or if there is reasonable suspicion of possession of contraband or the presence of other risk that could be used to cause harm to self or others; (e) Be free of any sexual harassment; (f) Be free of exploitation, including physical and financial exploitation.				
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(g) Have all clinical and personal information treated in accord with state and federal confidentiality regulations.				
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Item #2 – The right to have reasonable access to a telephone

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
RCW 71.05.217 Rights—Posting of list. (1) Insofar as danger to the individual or others is not created, each person involuntarily detained, treated in a less restrictive alternative course of treatment, or committed for treatment and evaluation pursuant to this chapter shall have, in addition to other rights not specifically withheld by law, the following rights, a list of which shall be prominently posted in all facilities, institutions, and hospitals providing such services: (e) To have reasonable access to a telephone, both to make and receive confidential calls;	Policy TXMIL 22 Telephone, Video Conferencing, Calls and Mail will be updated. Any calls a resident is requesting to make to CPS, law enforcement, DRW, or DOH; staff will make reasonable accommodation the confidential call. This change to policy will be communicated to the leadership team to implement. Staff will be trained in this policy and review the resident rights.	6/30/24 7/15/24	Executive Director of Intensive Youth Services.	QI Coordinator will review the procedure by 8/1/24 QI Coordinator will review five phone calls made by residents that took place between 7/20/24 and 8/20/24 to determine if procedure was followed. By 8/30/24 RTF Policies are reviewed every two years.

Item #3 – The right to be treated with dignity and respect

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0600 Individual rights. (1) Each behavioral health agency must protect and promote individual participant rights applicable to the services the agency is certified to	Any time there is an allegation of sexualized and/or inappropriate behavior, the staff is immediately placed on administrative leave and reported to Human Resources to gather	Process in place 5/1/24	Executive Director of Intensive Youth Services.	QI Coordinator will review PYR incident reports made between 6/1/24 and 6/14/24 to ensure the

provide in compliance with this chapter, and chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (d) Be treated with respect, dignity and privacy, except that staff may conduct reasonable searches to detect and prevent possession or use of contraband on the premises or to address risk of harm to the individual or others. "Reasonable" is defined as minimally invasive searches to detect contraband or invasive searches only upon the initial intake process or if there is reasonable suspicion of possession of contraband or the presence of other risk that could be used to cause harm to self or others;	more information. PYR will determine which outside entity needs to be reported to (DOH, Law Enforcement, CPS) and will determine the next steps. The process will be documented with Human Resources and the PYR program.	Document the process by 5/31/24	process is being followed by 6/15/24 QI Coordinator will meet with ED of Intensive Youth Services and the PYR Director to review process by 2/15/25
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WAC 246-341-0600 Individual rights. (1) Each behavioral health agency must protect and promote individual participant rights applicable to the services the agency is certified to provide in compliance with this chapter, and chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (d) Be treated with respect, dignity and privacy, except that staff may conduct reasonable searches to detect and prevent possession or use of contraband on the premises or to address risk of harm to the individual or others. "Reasonable" is defined as minimally invasive searches to detect contraband or invasive searches only upon the initial intake process or if there is reasonable suspicion of possession of contraband or the presence of other risk that could be used to cause harm to self or others;	Pearl Youth Residence has added training in "Boundaries" & "Ethics" training specific to PYR. All new staff will be trained within their on-boarding (3 classroom days and 4 shadow shifts before working a shift) and annually after.	Complete April 2024	ED of intensive Youth Services PYR Director	By 10/15/24, QI Coordinator will review completion of on-board training to determine if "Boundaries" and "Ethics" has been completed and report to the ED of Intensive Youth Services. QI Coordinator will meet with ED of Intensive Youth Services and the PYR Director to review onboarding training process by 2/15/25
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Item #4 – The right for all clinical and personal information to be treated in accordance with state and federal confidentiality regulations.

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0425 Agency administration—Individual service record system. Each behavioral health agency must: (1) Maintain a comprehensive individual service record system that includes policies and procedures that protect an individual's personal health information; (2) Ensure that the individual's personal health information is shared or released only in compliance with applicable state and federal law; (3) If maintaining electronic individual service records: (a) Provide secure, limited access through means that prevent modification or deletion after initial preparation; (b) Provide for a backup of records in the event of equipment, media, or human error; (c) Provide for protection from unauthorized access, including network and internet access; (d) Provide that each entry made in an individual's individual service record clearly identifies the author and who approved the entry, if applicable; and (e) Prohibit agency employees from using another employee's credentials to access, author, modify, or delete an entry from an individual's individual service record;	IT 01 was reviewed and updated 3/10/24 (attached)	Complete 3/10/24	Chief Technology Officer	CLR agency policies are reviewed at least every three years.

WAC 246-341-0425 Agency administration—Individual service record system. Each behavioral health agency must: (1) Maintain a comprehensive individual service record system that includes policies and procedures that protect an individual's personal health information; (2) Ensure that the individual's personal health information is shared or released only in compliance with applicable state and federal law; (3) If maintaining electronic individual service records: (a) Provide secure, limited access through means that prevent modification or deletion after initial preparation; (b) Provide for a backup of records in the event of equipment, media, or human error; (c) Provide for protection from unauthorized access, including network and internet access; (d) Provide that each entry made in an individual's individual service record clearly identifies the author and who approved the entry, if applicable; and (e) Prohibit agency employees from using another employee's credentials to access, author, modify, or delete an entry from an individual's individual service record;	All PYR staff are trained in HIPAA and Information Security, at hire and annually. To log in to staff's mobile device, there are security measures in place to ensure the intended user is the one accessing client health information (with Log in and two factor authentication)	Complete 5/1/24	Chief Technology Officer	Our HIT department will stay updated in health information security.
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Item #1 – Adherence to administrative, personnel and clinical policies.

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The administrator retains overall responsibility for responsibilities delegated to appropriate staff. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations; (b) There is sufficient qualified personnel to provide adequate treatment services and facility security; (c) All persons providing clinical services are appropriately credentialed for the clinical services they provide; (d) Clinical supervision of all clinical services including clinical services provided by trainees, students, and	Reporting Grid (circumstances to report and timeframes) has been updated to include CPS (under new regulations) (see attachment: Reporting Grid 2.24)	Complete February 2024	ED of Intensive Youth Services	By 6/15/24, QI Coordinator will review the timeliness of reports for a randomly selected two week period and report to the ED of Intensive Youth Services.

volunteers; (g) A written internal quality management plan, human resources plan or similarly specialized plan, as appropriate, is developed and maintained that: (i) Addresses the clinical supervision and training of staff providing clinical services; (ii) Monitors compliance with the rules in this chapter, and other state and federal rules and laws that govern agency licensing and certification requirements; and (iii) Continuously improves the quality of care in all of the following: (B) Use of evidence-based and promising practices; and (C) In response to critical incidents and substantiated complaints.				
RCW 71.05.217 Rights—Posting of list. (1) Insofar as danger to the individual or others is not created, each person involuntarily detained, treated in a less restrictive alternative course of treatment, or committed for treatment and evaluation pursuant to this chapter shall have, in addition to other rights not specifically withheld by law, the following rights, a list of which shall be prominently posted in all facilities, institutions, and hospitals providing such services: (e) To have reasonable access to a telephone, both to make and receive confidential calls;	Policy TXMIL 22 Telephone, Video Conferencing, Calls and Mail will be updated. Any calls a resident is requesting to make to CPS, law enforcement, DRW, or DOH; staff will make reasonable accommodation the confidential call. This change to policy will be communicated to the leadership team to implement. Staff will be trained in this policy and review the resident rights.	6/30/24 7/15/24	Executive Director of Intensive Youth Services.	QI Coordinator will review the procedure by 8/1/24 QI Coordinator will review five phone calls made by residents that took place between 7/20/24 and 8/20/24 to determine if procedure was followed. By 8/30/24 RTF Policies are reviewed every two years.

WAC 246-341-0600 Individual rights. (1) Each behavioral health agency must protect and promote individual participant rights applicable to the services the agency is certified to provide in compliance with this chapter, and chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (d) Be treated with respect, dignity and privacy, except that staff may conduct reasonable searches to detect and prevent possession or use of contraband on the premises or to address risk of harm to the individual or others. "Reasonable" is defined as minimally invasive searches to detect contraband or invasive searches only upon the initial intake process or if there is reasonable suspicion of possession of contraband or the presence of other risk that could be used to cause harm to self or others;	Pearl Youth Residence has added training in "Boundaries" & "Ethics" training specific to PYR. All new staff will be trained within their on-boarding (3 classroom days and 4 shadow shifts before working a shift) and annually after.	Complete April 2024	ED of Intensive Youth Services PYR Director	By 10/15/24, QI Coordinator will review completion of on-board training to determine if "Boundaries" and "Ethics" has been completed and report to the ED of Intensive Youth Services. QI Coordinator will meet with ED of Intensive Youth Services and the PYR Director to review onboarding training process by 2/15/25
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Item #2 – Clinical Supervision of clinical staff

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The administrator retains overall responsibility for responsibilities delegated to appropriate staff. (3) The administrator must delegate to a staff person the duty and responsibility to act on the administrator's behalf when the administrator is not on duty or on call. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations; (b) There is sufficient qualified personnel to provide adequate treatment services	PYR has hired three more unit supervisors, a total of four swing shift supervisors, two day shift and one overnight supervisor.	Complete March 15, 2024	PYR Director	PYR Director will work to hire and train new staff and provide clinical supervision of clinical staff

and facility security; (c) All persons providing clinical services are appropriately credentialed for the clinical services they provide; (d) Clinical supervision of all clinical services including clinical services provided by trainees, students, and volunteers; (e) There is an up-to-date personnel file for each employee, trainee, student, volunteer, and for each contracted staff person who provides or supervises an individual's care; (f) Personnel records document that Washington state patrol background checks consistent with chapter 43.43 RCW have been completed for each employee in contact with individuals receiving services; and (g) A written internal quality management plan, human resources plan or similarly specialized plan, as appropriate, is developed and maintained that: (i) Addresses the clinical supervision and training of staff providing clinical services; (ii) Monitors compliance with the rules in this chapter, and other state and federal rules and laws that govern agency licensing and certification requirements; and (iii) Continuously improves the quality of care in all of the following: (A) Cultural competency that aligns with the agency's local community and individuals the agency serves or may serve; (B) Use of evidence based and promising				
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practices; and (C) In response to critical incidents and substantiated complaints.				
WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The administrator retains overall responsibility for responsibilities delegated to appropriate staff. (3) The administrator must delegate to a staff person the duty and responsibility to act on the administrator's behalf when the administrator is not on duty or on call. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations; (b) There is sufficient qualified personnel to provide adequate treatment services and facility security; (c) All persons providing clinical services are appropriately credentialed for the clinical services they provide;	The supervisor team is working on a structured supervision plan that can be sustained to meet the staff's needs and schedules. A procedure for formal supervision will be developed.	8/15/24	ED of Intensive Youth Services PYR Director	By 9/15/24 QI Coordinator will meet with Director to read a sample of supervision notes and to determine if any follow up is needed. QI Coordinator will meet with ED of Intensive Youth Services and the PYR Director to review supervision process by 2/15/25

(d) Clinical supervision of all clinical services including clinical services provided by trainees, students, and volunteers; (e) There is an up-to-date personnel file for each employee, trainee, student, volunteer, and for each contracted staff person who provides or supervises an individual's care; (f) Personnel records document that Washington state patrol background checks consistent with chapter 43.43 RCW have been completed for each employee in contact with individuals receiving services; and (g) A written internal quality management plan, human resources plan or similarly specialized plan, as appropriate, is developed and maintained that: (i) Addresses the clinical supervision and training of staff providing clinical services; (ii) Monitors compliance with the rules in this chapter, and other state and federal rules and laws that govern agency licensing and certification requirements; and (iii) Continuously improves the quality of care in all of the following: (A) Cultural competency that aligns with the agency's local community and individuals the agency serves or may serve; (B) Use of evidence based and promising practices; and (C) In response to critical incidents and substantiated complaints.				
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WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The administrator retains overall responsibility for responsibilities delegated to appropriate staff.	Policy AD-14 Employee Use of Social Media (attached)	Complete 12/18/23	Chief Communications & Community Affairs Officer	Agency policies are reviewed at least every three years
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Item #3 – Ethical standards to improve the quality of patient care.

	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The	A Procedure will be written for staff taking residents on outings. The policy, TXMIL 13 (Out of Building Activities) will be updated to add that BHT staff need to be employed at CLR for at least 6 months to take a resident on one-on-one outings. Changes to the policy will be communicated to the leadership team to implement.	6/30/24 7/15/24	Executive Director of Intensive Youth Services.	QI Coordinator will review the procedure by 8/1/24 QI Coordinator will review five outings that took place between 7/20/24 and 8/20/24 to determine if procedure was followed. By 8/30/24 PYR RTF policies will be reviewed at least every two years.

administrator retains overall responsibility for responsibilities delegated to appropriate staff. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations; (b) There is sufficient qualified personnel to provide adequate treatment services and facility security; (c) All persons providing clinical services are appropriately credentialed for the clinical services they provide; (d) Clinical supervision of all clinical services including clinical services provided by trainees, students, and volunteers; (g) A written internal quality management plan, human resources plan or similarly specialized plan, as appropriate, is developed and maintained that: (i) Addresses the clinical supervision and training of staff providing clinical services; (ii) Monitors compliance with the rules in this chapter, and other state and federal rules and laws that govern agency licensing and certification requirements; and (iii) Continuously improves the quality of care in all of the following: (B) Use of evidence-based and promising practices; and (C) In response to critical incidents and substantiated complaints.				
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WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations; (b) There is sufficient qualified personnel to provide adequate treatment services and facility security; (c) All persons providing clinical services are appropriately credentialed for the clinical services they provide; (d) Clinical supervision of all clinical services including clinical services provided by trainees, students, and volunteers; (g) A written internal quality management plan, human resources plan or similarly specialized plan, as appropriate, is developed and maintained that: (i) Addresses the clinical supervision and training of staff providing clinical services; (ii) Monitors compliance with the rules	The supervisor team is working on a structured supervision plan that can be sustained to meet the staff's needs and schedules. A procedure for formal supervision will be developed.	8/15/24	PYR Director	By 9/15/24 QI Coordinator will meet with Director to read a sample of supervision notes and to determine if any follow up is needed. QI Coordinator will meet with ED of Intensive Youth Services and the PYR Director to review supervision process by 2/15/25
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in this chapter, and other state and federal rules and laws that govern agency licensing and certification requirements; and (iii) Continuously improves the quality of care in all of the following: (B) Use of evidence-based and promising practices; and (C) In response to critical incidents and substantiated complaints.				
WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters; (b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards.	The QI Coordinator will conduct a Quality survey of all residents that are willing to respond and compile results to send to the Executive Director of Intensive Youth Services.	9/15/24	Quality Improvement Coordinator Follow up by PYR Director & ED of Intensive Youth Services	Meeting will occur with the Director and ED by 9/30/24
WAC 246-341-0640 Individual service record content. A behavioral health agency is responsible for the components and documentation in an individual's individual service record content unless specified otherwise in certification or individual service requirements.	As an agency, Comprehensive Life Resources is aligned with WAC 246-341-0640 and does random checks for signatures regularly. When there is a change in position, there will be a transfer request ticket to alert the Administrative Services Manager to change the title in Extended Reach (Electronic Health Record) to make relevant updates.	Complete 3/1/24	PYR Director	QI Coordinator will do a random check of 5 current charts for at least 10 documents noting if the person completing the document has their title and credential with their name by October 15, 2024

Item #1 – Treatment Plans

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0640 Individual service record content. A behavioral health agency is responsible for the components and documentation in an individual's individual service record content unless specified otherwise in certification or individual service requirements. (1) The individual service record must include: (d) Individual service plan that: (i) Is completed or approved by a person appropriately credentialed or qualified to provide mental health, substance use, co-occurring, or problem gambling disorder services; (ii) Addresses issues identified in the assessment and by the individual or, if applicable, the individual's parent(s) or legal representative; (iii) Contains measurable goals or objectives and interventions; (iv) Must be mutually agreed upon and updated to address changes in identified needs and achievement of goals or at the request of the individual or, if applicable, the individual's parent or legal representative; (v) Must be in a terminology that is understandable to the individuals and the individual's family or legal representative, if applicable. WAC 246-337-100 Resident's individual service plan.	QI Coordinator will review five treatment plans created in the past 30 days and review with the criteria of 246-341-0640 and 246-337-100 and report findings to the Director and Executive Director to determine if any training needs to occur.	By 5/31/24	QI Coordinator	QI Coordinator will review five treatment plans created in the past 30 days and review with the criteria of 246-341-0640 and 246-337-100 and report findings to the Director and Executive Director to determine if any training needs to occur. Again by 10/15/24

This section does not apply to an RTF in its licensed capacity to provide pediatric transitional care services according to this chapter. (1) The licensee must develop and implement an individual service plan for each resident based on the resident's: (a) Initial health on admission; and (b) Health assessment(s). (2) Individual service plans must: (a) Be prepared by one or more staff involved in the resident's care with participation by the resident and by either his or her personal representative or parent or guardian when minors are involved; (b) Address the needs of a mother and baby during pregnancy and after delivery, if applicable; (c) Include work assignments given to a resident as part of their individual service plan, if applicable; (d) Be updated as additional needs are identified during treatment; and (e) Include a discharge health care plan.				
42 CFR part 2	Resident #2 Chart 120158 did receive SUD treatment. Due to 42 CFR part 2, we cannot release this information. We can share this live if needed.			
WAC 246-341-0410 Agency administration—Administrator key responsibilities. (1) The agency administrator is responsible for the day-to-day operation of the agency's provision of certified behavioral health treatment services, including: (a) All administrative matters;	The Clinical Manager will be reminded to check that all fields are complete before signing off on treatment plans.	By 5/15/24	Clinical Manager	QI Coordinator will review five treatment plans created in the past 30 days for diagnostic criteria and report findings to the Director and Executive Director to determine if any

(b) Individual care services; and (c) Meeting all applicable rules, policies, and ethical standards. (2) The administrator may delegate the responsibilities assigned to them under this section to appropriate staff. The administrator retains overall responsibility for responsibilities delegated to appropriate staff. (4) The administrator or their designee must ensure: (a) Administrative, personnel, and clinical policies and procedures are adhered to and compliant with the rules in this chapter and other applicable state and federal statutes and regulations; (b) There is sufficient qualified personnel to provide adequate treatment services and facility security; (c) All persons providing clinical services are appropriately credentialed for the clinical services they provide; (d) Clinical supervision of all clinical services including clinical services provided by trainees, students, and volunteers; (g) A written internal quality management plan, human resources plan or similarly specialized plan, as appropriate, is developed and maintained that: (i) Addresses the clinical supervision and training of staff providing clinical services; (ii) Monitors compliance with the rules in this chapter, and other state and federal rules and laws that govern agency licensing and certification requirements; and (iii) Continuously improves the quality of				training needs to occur. Again by 10/15/24
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care in all of the following: (B) Use of evidence-based and promising practices; and (C) In response to critical incidents and substantiated complaints.				
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Item #2 – Documentation of mandatory reporting events

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0640 Individual service record content. A behavioral health agency is responsible for the components and documentation in an individual's individual service record content unless specified otherwise in certification or individual service requirements. (2) When the following situations apply, the individual service record must include: (a) Documentation of confidential information that has been released without the consent of the individual under: (i) RCW 70.02.050; (ii) The Health Insurance Portability and Accountability Act (HIPAA); (iii) RCW 70.02.230 and 70.02.240 if the individual received mental health treatment services; and (iv) 42 C.F.R. Part 2. (b) Documentation that any mandatory reporting of abuse, neglect, or exploitation consistent with chapters 26.44 and 74.34 RCW has occurred.	Reporting Grid (circumstances to report and timeframes) has been updated to include CPS (under new regulations) (see attachment: Reporting Grid 2.2024)	Complete February 2024	QI Coordinator	By 6/15/24, QI Coordinator will review the timeliness of reports for a randomly selected two week period and report to the ED of Intensive Youth Services. By 9/15/24, QI Coordinator will review a random sample of three charts for the month of August (progress notes and therapy notes) to determine if a report of abuse, neglect, or exploitation was made to staff, there was a report to CPS also.

WAC 246-341-0510 Personnel—Agency record requirements. A behavioral health agency must maintain a personnel record for each person employed by the agency. (1) The personnel record must contain all of the following: (a) A signed position description. (b) A signed and dated commitment to maintain patient (individual) confidentiality in accordance with state and federal confidentiality requirements. (c) A record of an orientation to the agency within 90 days of hire that includes all of the following: (i) An overview of the agency's policies and procedures. (ii) Staff ethical standards and conduct, including reporting of unprofessional conduct to appropriate authorities.	The CLR Incident Reporting training is required for all CLR staff to complete annually. Boundaries and Ethics training instructs staff to reach out to their supervisor when they hear an allegation from a resident.	complete	Comprehensive Life Resources HR Director	HR will track trainings
WAC 246-341-0640 Individual service record content. A behavioral health agency is responsible for the components and documentation in an individual's individual service record content unless specified otherwise in certification or individual service requirements. (2) When the following situations apply, the individual service record must include: (b) Documentation that any mandatory reporting of abuse, neglect, or exploitation consistent with chapters 26.44 and 74.34 RCW has occurred.	Reports of abuse, neglect, or exploitation are called into CPS (and law enforcement, when necessary) documented in incident reports. Following WAC 246-341-0640 (2)(b), PYR is developing a system to document in the individual service record report follow-up (including grievances, communication with parent..), in a confidential way.	Reports process is complete Documentation process by 5/31/24	Director and Ex. Director	By 9/15/24, QI Coordinator will review a random sample of three charts for the month of August (progress notes and therapy notes) to determine if a report of abuse, neglect, or exploitation was made to staff, there was a report to CPS also and documented as the protocol indicates.

Item #3 – Court Orders

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0640 Individual service record content. A behavioral health agency is responsible for the components and documentation in an individual's individual service record content unless specified otherwise in certification or individual service requirements. (1) The individual service record must include: (d) Individual service plan that: (ii) Addresses issues identified in the assessment and by the individual or, if applicable, the individual's parent(s) or legal representative; (iii) Contains measurable goals or objectives and interventions; Based on interview and document review, the facility failed to ensure that the individual service record contained all required components and documentation. Failure to ensure that the individual service record is complete and contains all required documentation may lead to vulnerable patients continuing to be subjected to harmful situations; legal complications for patient receiving court ordered treatment; or, causing patients to feel a diminished trust in the services that are being provided if treatment plans or mandated reporting requirements aren't followed. In addition, failure to ensure that patient encounters are documented in such a manner that clinical staff can rely on their	The ITA court orders for the residents' charts being requested had the ITA court orders at the original time of request and the QI Coordinator had not specifically asked the Health Information and Technology department to upload the documents. QI Coordinator will request from HIT department to upload ITA Documents to the DOH ShareFile by 5/16/24. These can be found in the individual folders.	By 5/16/24	QI Coordinator	QI Coordinator will check back on all PYR clients that have an ITA court order to see that it is in their charts on 9/15/24 and 11/15/24. After that checks will be a part of the normal chart reviews.

independent recollection of the date; the time; the duration; and clinically significant behaviors demonstrated by the patient, may also lead to undesirable patient outcomes, including: diminished standards of patient care; a failure to capture clinically relevant details of patient (iv) Must be mutually agreed upon and updated to address changes in identified needs and achievement of goals or at the request of the individual or, if applicable, the individual's parent or legal representative; (2) When the following situations apply, the individual service record must include: (b) Documentation that any mandatory reporting of abuse, neglect, or exploitation consistent with chapters 26.44 and 74.34 RCW has occurred. (c) If treatment is court-ordered, a copy of the order. (f) Progress and group notes including the date, time, duration, participant's name, response to interventions or clinically significant behaviors during the group session, and a brief summary of the individual or group session and the name and credential of the staff member who provided it.				
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Item #4 – Progress notes capturing responses to interventions or clinically significant behaviors.

Rule Reference	Correction	Implementation Date	Responsible Party	Maintaining Correction
WAC 246-341-0640 Individual service record content. WAC 246-341-0600 Individual Rights.	The resident handbook will be updated. The staff and residents will be updated and a copy will be made available.	By 8/15/24	Admin. Services Manager and Director	QI Coordinator will review five charts between the dates of 9/1/24-10/15/24 to determine if clients are being offered a copy of the handbook. QI Coordinator will report findings to the Director and Executive Director to determine if any training needs to occur. By 10/15/24
WAC 246-341-0600 Individual rights. (1) Each behavioral health agency must protect and promote individual participant rights applicable to the services the agency is certified to provide in compliance with this chapter, and chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this	Demand for Discharge process has been updated 10/2023 (discharge form, speak with legal guardian, counselor) They are offered to work on plan for home and services in the community. This includes family-initiated treatment, when DCR is called. The clients continue in the regular milieu programming and not isolated in their room.	October 2023	Multi-Disciplinary Team	QI Coordinator will meet with ED of Intensive Youth Services and PYR Director to review the Demand for Discharge process and review the process of the past two clients that had asked for an early discharge. By 9/15/24 And again 2/15/25 They will determine if any changes to the process need to occur.

subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements.				
WAC 246-341-0600 Individual rights. (1) Each behavioral health agency must protect and promote individual participant rights applicable to the services the agency is certified to provide in compliance with this chapter, and chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. (2) Each agency must develop a statement of individual participant rights applicable to the services the agency is certified to provide, to ensure an individual's rights are protected in compliance with chapters 70.41, 71.05, 71.12, 71.24, and 71.34 RCW, as applicable. To the extent that the rights set out in those chapters do not specifically address the rights in this subsection or are not applicable to all of the agency's services, the agency must develop a general statement of individual participant rights that incorporates at a minimum the following statements. "You have the right to:" (d) Be treated with respect, dignity and privacy, except that staff may conduct reasonable searches to detect and prevent possession or use of contraband on the premises or to address risk of harm to the individual or others. "Reasonable" is defined as	There is a more consistent guideline for what consequences are earned. The PYR staff receive training in Discipline and Milieu programming. This includes scenarios and descriptions and what the requirements are. There is a flow chart for BIRs and IRs (involving nursing, when to write an Incident Report...) The consequences have manager approval. There is a document to track and review off-programming and the Director monitors and reports trends to the Executive Director.	October 2023 training Manager approval was added March 2024 Tracking Form May 2024	Milieu Managers, Director	The tracking will be reviewed at least quarterly.

minimally invasive searches to detect contraband or invasive searches only upon the initial intake process or if there is reasonable suspicion of possession of contraband or the presence of other risk that could be used to cause harm to self or others; (e) Be free of any sexual harassment; (f) Be free of exploitation, including physical and financial exploitation; (g) Have all clinical and personal information treated in accord with state and federal confidentiality regulations; (h) Participate in the development of your individual service plan and receive a copy of the plan if desired; (i) Make a mental health advance directive consistent with chapter 71.32 RCW; ... (k) Submit a report to the department when you feel the agency has violated your rights or a WAC requirement regulating behavioral health agencies. ... (d) Posted in public areas; and (e) Available to any participant upon request. (4) At the time of admission and upon client request, the agency must provide each client with information on how to file a report to the department if they feel their rights or requirements of this chapter have been violated.				
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STATE OF WASHINGTON
DEPARTMENT OF HEALTH

07/30/2025

Pearl Youth Residence
1201 S Proctor St
Tacoma, WA 98405-2047

Dear Kymm Dozal,

The Department of Health has reviewed and verified the completion of the Directed Plan of Correction (DPOC) issued on December 10, 2024, pursuant to WAC 246-337-021, related to the facility's identified noncompliance. I appreciate your timely cooperation and commitment to addressing the required corrective actions.

Following the review of the submitted documentation, policies and procedures specific to patient rights, seclusion and restraint, patient confidentiality, patient care services, mandatory reporting, safety and security, governance and administration, and quality improvement, and an onsite review, it has been determined that the facility has fulfilled all required elements of the DPOC. Please be reminded that while this approval reflects the facility's compliance with the outlined plan, ongoing compliance with applicable statutes and regulations remains required. The Department will continue to monitor the provision of behavioral health services through the routine oversight process.

Thank you for your attention to this matter and for your continued commitment to providing safe and effective behavioral health services. If you have any questions or need further clarification, please do not hesitate to contact me directly.

/s/Sean McKinney

Sean McKinney (He/Him)
Behavioral Health Investigator
Office of Health Systems Oversight
Health Systems Quality Assurance
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